

of the Government of Georgia

Resolution No. 459

October 6, 2017

St. Tbilisi

Regarding the approval of the 2017-2030 national strategy for the promotion of maternal and newborn health of Georgia and the 2017-2019 action plan for its implementation, Article 1, Articles 132 and 133 of the Law of

Georgia "On Health Protection" and "The Structure, Powers and Activity of the Government of Georgia" In accordance with Article 6 of the Law of Georgia, approve the attached 2017-2030 national strategy for the promotion of maternal and newborn health of Georgia and the 2017-2019 action plan for its implementation. Article 2 The resolution shall take effect immediately after its publication.

Prime Minister

Giorgi Kvirikashvili

2017-2030 national strategy for promotion of maternal and newborn health of Georgia

Foreword

Improving the health of mothers and newborns is an important priority of the country for the next 14 years. For this purpose, the government has developed a long-term strategy for 2017-2030 and a three-year action plan for its implementation (2017-2019). One of the main prerequisites for maternal and newborn health is the

provision of quality family planning services, as well as the sexual and reproductive health of young people. That is why the mentioned directions were also reflected in the strategic document.

The goal of the Maternal and Newborn Health Promotion Strategy is to maintain and expand evidence-based and effective interventions to ensure the survival and health care of mothers and newborns.

The document defines a strategic framework and interventions to improve the areas of maternal and newborn health, family planning and reproductive health. It is harmonized with recently developed international strategic documents: the UN Sustainable Development Goals and Targets (2015), the European Bureau of the World Health Organization (WHO) "Health and Well-Being European Policy - Health 2020" (2012) and the World Health Organization (WHO) Europe Bureau of Sexual and Reproductive Health Development Action Plan (2016).

Despite significant progress over the past two decades in meeting international targets, the country still faces significant challenges in maternal and newborn health, family planning, and youth sexual and reproductive health. The strategy document analyzes the main determinants of these challenges and

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Various priority interventions needed to overcome them are given.

The strategy document includes the following three directions and their corresponding tasks for 2030:

Promotion of maternal and newborn health:

Goal 1: All women will have universal access to evidence-based prenatal, antenatal, obstetric, neonatal and postpartum care;

Task 2: the quality of medical services for mothers and newborns will be improved, services will be integrated and standardized;

Task 3: awareness and knowledge of the population about healthy behavior, high-quality medical service standards and patients' rights will be significantly improved.

Family Planning:

Task 4: Family planning services will be readily available to all persons with appropriate needs;

Task 5: The quality of family planning services will meet international standards.

Sexual and reproductive health of young people:

Task 6: Age-appropriate education on sexual and reproductive health issues will be fully accessible to young people;

Goal 7: Young people will have full access to reproductive health services.

Table of Contents

Preface.

table of contents

1. Introduction.

1.1. Foundations of the National Strategy for the Promotion of Maternal and Newborn Health

1.2. Areas of strategy application

1.3. Vision

1.4. purpose

1.5. target indicators

1.6. tasks

1.7. guiding principles

2. Maternal and newborn health

2.1. Epidemiology of maternal and newborn health

2.3. Tasks and priority interventions for improving the health of mothers and newborns in Georgia

3. Family planning

3.1. Development of family planning and its current situation in Georgia

3.2. Priority interventions to improve family planning services in Georgia

4. Sexual and reproductive health of young people

4.1. Current state of sexual and reproductive health of young people in Georgia

4.2. Approaches and priority interventions to improve the sexual and reproductive health of young people

5. Implementation and management mechanisms

6. Risk assessment

7. Monitoring and evaluation

8 2017-2019 Action Plan for the Implementation of the National Strategy for the Promotion of Health Protection of Mothers and Newborns of Georgia

8.1. Introduction

8.2. guiding principles

8.3. Vision, purpose and strategic objectives

8.4. strategic tasks

8.5. Achievable targets for 2020 and 2030

8.6. Stakeholder Responsibilities and Functions

8.7. Funding of measures to promote the health of mothers and newborns

8.8. Measures for the implementation of the National Strategy for the Promotion of Health Protection of Mothers and Newborns of Georgia for 2017-2019

9. Literature

1. Introduction

1.1. Foundations of the National Strategy for the Promotion of Maternal and Newborn Health

This document presents long-term, necessary measures until 2030, the purpose of which is to improve the health of mothers and newborns and closely related family planning and reproductive health issues in Georgia.

In 2014, the action program adopted at the International Conference on Population and Development (ICPD) in Cairo in 1994 was completed, and in 2015, the UN Millennium Development Goals (MDGs) were completed. In 2000, Georgia, as a signatory country of the UN Millennium Declaration, set its own targets to be met in 2015 in relation to health-related goals. [1]

As a result of effective initiatives and significant efforts made by the Government of Georgia regarding maternal and child health, Georgia exceeded the 4th Millennium Development Goal and reduced the under-five mortality rate from 48 (in 1990) to 12 (in 2015) per 1,000 live births. , instead of target 16. In the same period, infant mortality per 1,000 live births decreased from 41 to 11, and neonatal mortality decreased from 25 to 7[2] . However, it was not possible to fulfill the 5th goal - to reduce maternal mortality by three quarters. According to the United Nations Inter-Agency Maternal Mortality Assessment Group (MMEIG), the maternal mortality rate in 2015 was 36 per 100,000 live births,[3] which is more than twice the national Millennium Development Goal target of 16. Despite the downward trend in recent years, maternal, infant and neonatal mortality rates are still higher than the average rates of the European region (respectively: 16, 5 and 3 in 20153).

According to reproductive health surveys, in 1995-2009, the contraceptive prevalence rate in Georgia increased from 41% to 54%, and the use of modern contraceptive methods increased from 20% to 35% (a 75% increase). [4] . According to routine statistics, during the last 10 years, the number of referrals for choosing a method of contraception in the institutions providing reproductive services in the country has almost tripled, but, despite such visible results, Georgia still has one of the lowest rates of contraceptive use in Eastern Europe[5] .

The Total Artificial Abortion Rate (TIAR) in Georgia has decreased significantly (from 3.7 in 1999 to 1.6 in 2010 per woman per lifetime), although it is twice the European average and one of the highest in the countries of the former Soviet Union. This indicates a high level of unsatisfied demand for modern family planning methods.

Little progress has been made in terms of the Millennium Development Goal 5b (adolescent fertility rate) and the Cairo International Conference on Population and Development Program of Action indicators for assessing sexual and reproductive health. In particular, in 2010-2012, there was a decreasing trend in the rate of pregnancy among teenagers (15-19 years old), but since 2013 it started to increase again and in 2015 it was 48.6[6] . An increase in the birth rate among adolescents since 2000 was observed in another country of the European region of the World Health Organization - Albania. In Western European countries, this figure ranges from 15 to 25.

Following the Millennium Development Goals, by adopting the internationally agreed targets [7], UN member states have reaffirmed their will

Ensure universal access to sexual reproductive health and care services, including family planning, population awareness and education, and integrate reproductive health issues into national strategies to target). In programs 2030 7th and

by the year" (3rd goal,

In 2015, the World Health Organization developed the "Global Strategy for the Promotion of Women's, Children's and Adolescent Health (2016-2030): Survival, Development, Transformation"[9] , which identified priority areas and evidence-based interventions for maternal and child health over the current 15 years. health and with him

Mostly related to improving reproductive health. Accordingly, for the UN member states, this document, along with the sustainable development goals, is a call and includes guidelines for the development of updated strategic tasks in this area[10] .

In 2016, the Parliament of Georgia adopted the concept of demographic security. It is based on national efforts, achievements and experiences in all sectors. Universal access to reproductive health services, information and education is one of the goals of this policy. The document also states that "in the conditions of the demographic regime in Georgia - increased life expectancy, high rate of emigration and relatively high birth rate, the most relevant priorities of the national health care system, from the point of view of birth and reproductive health, include: identifying various reproductive health risks and responding to them; improving quality reproductive health services, including services focused on youth, mothers and babies; prevent and improve management of complicated pregnancy and childbirth; With access to modern methods of family planning, artificial abortion can be avoided, breast cancer and uterine cancer can be prevented, diagnosis at an early stage and timely treatment." Some of these priorities are also adequately reflected in this strategy.

Finally, it should be noted that in September 2016, at the 66th session of the European Regional Committee of the World Health Organization, the European Reproductive Health Action Plan (2017-2021)[11] was adopted , which was developed in close cooperation with 53 member states of the region, and Georgia is one of them. The European Action Plan is linked to key policy documents relevant to this area[12] , such as the WHO European Bureau's Health 2020 Strategy,[13] the ICPD Action Program and the UN Sustainable Development Goals.

Georgia's maternal and newborn health promotion strategy fully shares the main values and provisions of the European action plan.

1.2. Areas of strategy application

The overall goal of the strategy is to maintain and expand evidence-based and effective interventions to reduce maternal and newborn morbidity and mortality; Strengthening reproductive health and ensuring universal/universal access to high-quality services.

The strategy outlines key directions and provides guidelines for important determinants of maternal and newborn health, such as family planning and adolescent sexual and reproductive health.

Reducing maternal and newborn mortality requires a coordinated and long-term effort. Many factors affect the health of women and newborns after pregnancy and childbirth. Increasingly, evidence is gaining recognition that high maternal and infant mortality rates are the result of significant challenges in the health sector. In addition, factors related to socio-cultural values, economic conditions of families and society, and national policies contribute to this.

Concomitant factors that also increase maternal and newborn mortality rates include delays in identifying problems, making decisions about health care, and accessing and receiving quality health care.

All the factors listed above constitute the so-called The main elements of the "three delays" described in Thaddeus and Maine's model[14] are still a real challenge facing pregnant women and newborns in Georgia. Therefore, to mobilize resources and gain public support, multisectoral, creative strategies and, of course, political will are needed to make changes at different levels and achieve sustainable improvements in maternal and newborn health.

At the International Conference on Population and Development (Cairo, 1994), the definition of reproductive health was adopted, which is still an international standard. The definition states that reproductive health includes three closely related areas: sexual health, family planning, and maternal and newborn health.

This definition became the starting point for the creation of the strategic document. According to the same guiding principle, family planning and adolescent sexual and reproductive health have been included in the present strategy for promoting maternal and newborn health. Family planning is not only the right of parents to decide how many children to have and at what intervals. It is also an important factor in reducing the burden of unintended pregnancies and, therefore, in reducing induced abortion and abortion-related morbidity and mortality. Unprotected sex among adolescents leads to sexually transmitted infections (STIs), or mostly unwanted pregnancies, most of which are more likely to end in artificial abortion, which may threaten the health of adolescents, especially among adolescents under 20 years of age [15] . It should be noted that relatively less attention was paid to family planning and sexual and reproductive health of adolescents in Georgia, which must change in the future.

Some areas of reproductive health (general promotion of sexual health; sexual and reproductive health of men; infertility; infections of the reproductive tract, cervical and breast cancer[16]) are not included in the strategy as a separate topic, because they are discussed in detail in other strategic documents (in Georgia, non-communicable diseases National Strategy for Prevention and Control 2017-2020), or up-to-date and evidence-based recommendations for policy and practice are available in these areas. As for the issue of artificial abortion, it is included to a certain extent in the chapter - "Family Planning".

The three-year action plan for the implementation of the strategy is a framework document for the implementation of specific interventions in the area of maternal and newborn and reproductive health/family planning over the next three years.

The mission of the strategy for the promotion of maternal and newborn health is: in the wake of the country's development, to coordinate the efforts expressed by the state and to increase the state support for maternal and newborn health/reproductive health/family planning issues.

- Ensuring the sustainability of the necessary human and financial resources to support maternal and newborn health/reproductive health/family planning programs and initiatives and consider them a national priority.
- Reducing the gap between planned and implemented measures in the field of maternal and newborn health/reproductive health/family planning and creating a favorable environment for their implementation.
- Development of performance indicators to evaluate the progress of strategy implementation.

1.3. Vision

By 2030, preventable maternal and newborn deaths will
Eliminating stillbirths, all children will be desirable and possible
All unwanted pregnancies will be avoided.

1.4. purpose

Sustaining and expanding evidence-based and effective interventions
in order to save mothers and newborns and protect their health; as well as
Strengthening the field of reproductive health directly related to them and
Ensuring universal access to high quality services.

1.5. target indicators

Reduce maternal mortality from 32 (2015) to 12 per 100,000 live births in 2030
for the year.

· Reduction of neonatal mortality from 6.1 (2015) to 5 per 1,000 live births, 2030
for the year.

· Reducing unmet need for family planning from 30.5% by 2030. (2010), to <15%,

· Reduction of the total artificial abortion rate from 1.6 (2010) to 0.5 by 2030.

· A decrease in the birth rate of adolescents (15-19 years) from 48.6 (2015) to <20 per 1,000 women. 2030
per year.

1.6. tasks

The objectives of the mentioned strategy for 2030 are:

Promotion of maternal and newborn health:

Goal 1: All women will have universal access to evidence-based prenatal care
antenatal, obstetric, neonatal and postpartum services;

Task 2: The quality of medical services for mothers and newborns will be improved
integration and standardization of services;

Task 3: The population's awareness and knowledge of healthy will be significantly improved
behavior, high quality medical service standards and patients' rights
about.

Family Planning:

Task 4: Family planning services are readily available to all concerned
for a person in need;

Task 5: The quality of family planning services will meet international standards.

Sexual and reproductive health of young people:

Task 6: Age-appropriate education will be fully accessible to young people
on sexual and reproductive health issues;

1.7. guiding principles

This strategy is closely related to the international strategic documents mentioned above, which are based on globally recognized principles relevant to the field of reproductive health:

human rights

The health of women, children and adolescents is recognized as a fundamental human right in several international agreements, namely: the International Covenant on Economic, Social and Cultural Rights (1966), the Convention on the Elimination of All Forms of Discrimination against Women (1979) and the Convention on the Rights of the Child (1989). The basis of the strategy in this direction is the existing treaties and obligations on human rights. It is also based on the Action Program of the International Conference on Population and Development (Cairo, 1994); The Beijing Declaration and Platform for Action agreed upon at the Fourth World Conference on Women (Beijing, 1995).

Health, within the framework of international law, is a human right that is closely related to other human rights and is inseparable from these rights. Key human rights interventions include policy and legislation, equality and non-discrimination, service delivery and participation; Also, determinants of health, socio-cultural, political and economic issues and accountability. In addition to meeting legal obligations, there is evidence that a human rights-based approach has a positive impact on the health of women, children and adolescents.

1. Gender equality and gender sensitivity

Gender equality is a necessary prerequisite for women to make informed choices and seek and receive the services they need and want for their health. Unequal gender norms and stereotypes create bias in politics and institutions, which has serious consequences for the effectiveness of services. Therefore, the elimination of discrimination is of fundamental importance. On the one hand, it is necessary to ensure that women and girls know their rights so that they can request services that are gender-sensitive and free from stigma and discrimination. On the other hand, it is necessary to strengthen the support of men and their involvement in achieving gender equality. Such an attitude contributes to the improvement of the sexual and reproductive health of women, men and young people and the realization of the declared rights.

Based on all of the above, the successful implementation of a reproductive health strategy in itself means an effective response to unequal gender norms.

2. Continuity of reproductive health throughout life

Sexual and reproductive health needs are not limited to the reproductive years. Attitudes towards the health needs and priorities of both men and women across the 'life span' contribute to or hinder a safe, satisfying sex life; Also, the ability of women to give birth to healthy children and at the same time maintain their own health.

3. Quality of care

Improving the quality of services is the most important challenge of the current health care system of Georgia. Existence of gaps related to the quality of service affects the cost-effectiveness and, ultimately, the consumption model of the services. Therefore, quality improvement should be an integral component of all programs focused on service delivery, while ensuring adherence to clinical guidelines, protocols and standards, confidentiality, security, technical sustainability, patient-friendly service delivery, accountability and continuity of care.

4. Other guiding principles

In parallel with the European Action Plan for Sexual and Reproductive Health, the following guiding principles are defined:

- Alignment with the 2020 health vision, policies and priorities (World Health Organization, Regional Bureau for Europe);
- evidence-based and results-oriented action;
- Main emphasis on disease prevention, health promotion and strengthening of public involvement in these processes;
- Person-centered care.

2. Maternal and newborn health

2.1. Epidemiology of maternal and newborn health

Worldwide, approximately 800 women and 7,700 newborns die every day from complications of pregnancy and childbirth and from various neonatal causes. In addition, 7,300 women's pregnancies ended in stillbirth.[18] A large proportion of maternal deaths[19] are still due to obstetric causes such as haemorrhage (27%), complications related to hypertension (14%) and sepsis (11%). The increase in the number of maternal deaths caused by indirect causes is alarming, accounting for more than half of maternal deaths [20] .

Infant mortality in the world[21] accounts for 44% of the mortality of children under 5 years of age. About 1 million newborns die on the day of birth, and about 2 million - in the first week of life. Globally, 40% of the estimated 2.64 million stillbirths (1.2 million per year) occur during childbirth. The main direct causes of neonatal mortality are: premature birth (35%), childbirth-related processes (24%) and severe infections (20%).[22] Almost 80% of neonatal deaths occur in infants weighing less than 2,500 grams at birth, especially those born prematurely.[23] Many preterm infants die within the first month, making preterm birth the leading cause of infant mortality worldwide , [24] and those who survive the first month of life after preterm birth are more likely to have disabilities for the rest of their lives.[25]

Evidence-based, cost-effective, and effective are already available

Interventions that make it possible to prevent a large proportion of maternal and newborn deaths. Preventable maternal and newborn mortality remains one of the world's most critical challenges in the new 2030 Agenda for Sustainable Development.[26]

In order to achieve the goals of sustainable development in Georgia, it is necessary to reduce maternal and newborn mortality at a faster rate. Maternal and infant mortality are considered the main indicators of population health and socio-economic development, and their reduction is one of the most important goals of public health in Georgia. It covers a wide range of indicators of health-related behavior and health systems functioning that affect the health, well-being and quality of life of women, children and families.

Health indicators of mothers and their children (including newborns) are closely related. Maternal mortality and morbidity affect infant and child survival, growth and development. Therefore, better coordination of interventions, integration of services across the continuum of care, and establishment of sustainable partnerships within and outside the health sector are essential parts of a national maternal and newborn health strategy.

During the last decades, Georgia has achieved great success in reducing maternal and newborn mortality, although this progress is relatively modest in the case of mothers. According to the United Nations Inter-Agency Maternal Mortality Assessment Group, the maternal mortality rate has decreased from 60 per 100,000 live births (2000) to 36 (2015); However, it is still twice the national target of the 5th Millennium Development Goal - 16 - and the average of the European region.

In the same period, based on the latest data from the United Nations Inter-Agency Group on Child Mortality Evaluation (IGME), the mortality rate of children under 5 years of age in Georgia decreased from 48 (1990) to 12 (2015) per 1,000 live births, and thus is significantly below the Millennium Development Goals. on the national target set by the goals - 16. Special progress has been made in reducing infant mortality (per 1,000 live births) - from 41 (1990) to 11 (2015) and reducing neonatal deaths (per 1,000 live births) - 25- from to 7[27] . Both of these rates were the lowest ever recorded in 2015, as was the officially reported stillbirth rate of 9.7 per 1,000 births (after 22 weeks' gestation).[28] Despite the decreasing trends, infant and neonatal deaths and stillbirths are still higher than the average indicators in the European region (5; 3 and 3.4 respectively)²⁵ . Georgia ranks sixth among post-Soviet countries in terms of neonatal mortality, although this rate is lower than in neighboring countries - Armenia, Azerbaijan, and Central Asian republics.[29] The proportion of neonatal mortality in both under-5 and infant mortality is 60% and 70%, respectively, and remains high. At the same time, the share of early neonatal mortality in the overall neonatal mortality in 2015 dropped to the lowest level in the last decade and amounted to 58%.²⁶

The main causes of maternal and newborn deaths in the country are largely consistent with the global causal model. ²⁶,[30] Although the immediate medical diagnoses of maternal, fetal and newborn deaths differ, the main causes of these deaths are very close to each other. Many diseases that cause complications for the mother during pregnancy, childbirth and the postpartum period also cause complications for the baby.

According to the official statistical data of Georgia, in 2015, the number of mothers

21 cases of death (direct and indirect causes), including 19 early (pregnancy or within 42 days after the end of pregnancy) and 2 late (between 43-365 days after the end of pregnancy). Maternal mortality rate was 32 per 100,000 live births (the rate was counted in 19 cases). Of the 19 cases of maternal death, 57.9% were caused by direct causes, 26.3% by indirect causes, and in 15.8% the cause could not be identified. The structure of maternal mortality is as follows: 21% bleeding during childbirth and postpartum period, 10.5% – infection, 5.3% – preeclampsia and 5.3% – obstetric embolism. It should be noted that with the introduction of evidence-based effective perinatal care approaches recommended by the World Health Organization in Georgia in 2006 (John Snow Institute/US Agency for International Development), the structure of the main direct causes of maternal death has significantly changed over the last decade. In particular, the share of postpartum hemorrhage in maternal mortality has decreased, which is largely the result of the introduction of active management of the third stage of labor in daily obstetric practice.[31] Similarly, post-abortion complications have declined, mainly as a result of a decline in the total abortion rate (from 3.1 in 2005 to 1.6 in 2010) and some improvements in post-abortion care, achieved by both government and donor organizations (UNFPA, through programs supported by the John Snow Institute/USAID).28,29

At the same time, sepsis remains an important cause of maternal mortality in Georgia. Factors that predispose women to postpartum infections include anemia, poor nutrition, prolonged labor with frequent vaginal examinations, and premature rupture of membranes.[32] A growing body of evidence points to caesarean section as one of the most important risk factors for the development of postpartum sepsis.[33] During the last decade, the caesarean section rate in Georgia increased by almost 50% and reached 41% in 2015, which is almost twice the rate in the European region and the similar rate in the Commonwealth of Independent States.28 The increase in the rate of primary caesarean section in Georgia is particularly worrying. More than half of the deliveries (55%) are performed without medical indications. 28

In the recent period, the world has paid special attention to the high incidence of indirect causes of death, which is described in the concept of "obstetric practice change"[34] , according to which, along with the decrease in fertility and maternal mortality, the main causes of maternal mortality are shifting to communicable and non-communicable diseases. Infectious diseases, including sexually transmitted infections and urinary tract infections, as well as non-infectious diseases (hypertension, diabetes) have serious consequences in terms of both maternal and newborn morbidity. In particular, the frequency of adverse outcomes such as prematurity, small size for gestational age, stillbirth and early neonatal mortality is increasing[35] .

Georgia needs to strengthen the identification and management of indirect causes of maternal mortality. Also, it is necessary to coordinate activities with different sectors and health service providers in order to carry out screening of communicable and non-communicable diseases, appropriate and timely treatment and to implement the recommendations/requirements of relevant clinical guidelines and protocols in practice.[36]

The main determinants of infant mortality and morbidity in Georgia are preterm birth, fetal growth retardation, and congenital anomalies, as well as external factors such as social, behavioral, and cultural determinants of health. Variables associated with preterm birth include the woman's age at pregnancy, multiple pregnancy, between births

Short interval cesarean sections performed with non-medical indications.

Evidence suggests that infants of adolescent mothers (<20 years of age) are more likely to be born low birth weight and/or premature, and have a 50% higher risk of perinatal death compared to children of adult mothers.[38]

Based on official data, the majority of neonatal deaths in Georgia in 2015 (75%) occurred among preterm infants, 71% of whom were premature for gestational age and 56% by birth weight (<2,500g).

Similarly, 71% of all stillbirths occurred at preterm birth (<37 weeks), mostly between 22-27 weeks of gestation (35%), and 26% occurred at term (37 weeks or later).

It should be noted that in 82% of the total stillbirths, fetal death occurred in the antenatal period, and 14% - during childbirth. By comparison, the perinatal stillbirth rate ranges from 4-10% of total stillbirths in developed countries.[39] Infections, preeclampsia/eclampsia, and hypertension contribute to stillbirth rates.[40],[41],[42] It is recognized that most effective interventions that improve maternal health and reduce maternal mortality also reduce rates of stillbirth and preterm birth. Specific measures to prevent stillbirth (both antenatal and intranatal fetal death) and preterm birth include: screening and management of sexually transmitted infections, high blood pressure and diabetes; educational activities for the purpose of stopping tobacco consumption; Targeted care for women at imminent risk of preterm birth, including appropriate management of preeclampsia/eclampsia; Enhanced antenatal surveillance, emergency obstetric care, appropriate use of induction of labor and caesarean section[43] , monitoring of antenatal corticosteroid use by women at risk of preterm delivery [44]

Prevention of congenital syphilis and mother-to-child transmission of HIV infection/AIDS is an important component of measures aimed at reducing maternal and newborn morbidity and mortality. The incidence of HIV infection/AIDS among pregnant women in Georgia is much lower (0.04%) than in the population as a whole (0.07% in 2013). Screening of all pregnant women for HIV infection/AIDS, provision of standard antiretroviral treatment for all HIV-positive pregnant women, preventive antiretroviral therapy for every newborn and their social protection, as well as improving the quality of the program for the prevention of mother-to-child transmission of HIV infection/AIDS - this is the 2016-2018 HIV infection/ The key priority interventions of the National AIDS Strategy are: Linking the maternal and newborn health program with HIV/AIDS and STD programs (screening for STD infections and free treatment of pregnant women with syphilis within the framework of the state maternal and child health program) will contribute to the coordination of preventive activities in the country to eliminate congenital syphilis, prevent new HIV infections in newborns and their mothers. in order to save lives.

To achieve a reduction in preventable maternal and newborn mortality, it is essential that high-quality health care and life-saving interventions are provided and accessible to all pregnant women and newborns. The investment made in the implementation of the mentioned interventions gives a triple effect. 41% of neonatal deaths and 70% of stillbirths can be prevented with effective interventions during and after birth[45]. At the same time, supporting the mother-child bond and integrating maternal health services with newborn health, family planning, communicable and noncommunicable diseases, and nutrition is a priority that

It will contribute to the implementation of cost-effective and patient-friendly services.

The main principle of achieving and maintaining the desired changes is to ensure the high quality of services provided in the pre-pregnancy, antenatal period, delivery and post-partum period along with continuous care[46].

On the other hand, maternal and newborn health programs should consider the impact of social determinants of health on maternal and newborn mortality. Poverty and inequality, in different ways, have a very negative impact on mothers and newborns. These include maternal malnutrition, inadequate housing and sanitation, low levels of education and gender discrimination. Health communication and behavior change interventions are essential to disseminate knowledge about maternal and newborn care and family planning, improve family behavior and health care seeking during potentially life-threatening complications, thereby reducing delays in seeking health care and preventable deaths. Through a program framework focused on meeting existing requirements, sustainable quality of services and a favorable environment, Georgia will promote the introduction of integrated strategies and services to improve the health of mothers and newborns.

2.2. Development and current situation of maternal and newborn healthcare in Georgia

According to the World Bank (2017), Georgia is a lower-middle-income country with a population of approximately 3.7 million. After the collapse of the Soviet system and gaining independence, significant health reforms were implemented in the country, with special emphasis on improving maternal and child health and increasing access to health services. However, despite progress made over the past 20 years, challenges remain in various areas related to maternal and newborn health.

1. Government and governance

On December 26, 2014, the Government of Georgia approved Resolution No. 724 "On the Approval of the State Concept of the Georgian Health Care System for 2014-2020 "Universal Health Care and Quality Management to Protect Patients' Rights", which includes such fundamental principles of the country's health care sector development as universal access, sustainability, cost-effective and transparent governance, and strengthening interagency cooperation.[47]

State policy in the field of health care is based on such basic values as protection of human rights and justice, which, among other directions, includes the elimination of inequality in terms of equal access to medical services and the right to participate in the decision-making process. The state policy in the field of health care takes into account both the epidemiological, social and economic reality of the country, as well as internationally recognized political declarations and action platforms in the field of health care. This concept gives priority to the health of mothers and children.

During the last decade, the government has made significant efforts to improve maternal and child health in the country. Several state-sponsored maternal and child health programs have been launched to provide antenatal care, detection and management of high-risk pregnancies, early detection of congenital anomalies, screening of pregnant women for HIV/AIDS, hepatitis B and C and syphilis, folic acid and iron supplementation for pregnant women. Food containing supplements and micronutrients for children aged 6-23 months

Nevertheless, the current state programs and the funds allocated to finance maternal and child health programs are mainly focused on individual specific directions. In addition, no service or patient satisfaction surveys are conducted at the level of medical facilities. It is necessary to develop and implement an integrated system of monitoring and evaluation of state programs.

Currently, the protection of patients' rights in terms of the quality of medical services is provided by the State Regulatory Agency for Medical Activities under the state control of the Ministry of Labor, Health and Social Protection of Georgia.

In general, in order to identify existing gaps and implement effective response mechanisms, it is necessary to regularly analyze the legislative environment and assess patients' needs. Licensing/permitting and technical regulation conditions for high-risk medical activities, as well as other regulatory norms should be revised, primarily in relation to infrastructure and human resources, to ensure compliance of the integrated model of medical services with internationally recognized criteria.

Also, it is necessary to closely cooperate with the Ministry of Education and Science of Georgia and implement coordinated measures regarding the issues of pre-diploma medical education.

2. Healthcare financing

During the last two decades, the healthcare financing system of Georgia has undergone a significant transformation. With the support of the Government of Georgia, in February 2013, the state program of universal healthcare was launched. The universal blueberry was based on a significant increase in public spending on health care (the share of public spending on health care in relation to the gross domestic product increased from 1.8% (in 2012) to 2.8% (in 2015)). The universal health program and other state health programs cover a wide range of services for the population of Georgia, including childbirth (both natural and caesarean) and medical services for children 0-18 years of age. The Universal Health Program is an important initiative in terms of ensuring equity, as well as improving access to and utilization of health services. As a result, the specific share of out-of-pocket payments for medical services decreased significantly - from 73.4% (in 2012) to 58.4% (in 2015[49]). The implementation of universal health care has had a significant impact on protecting Georgian citizens from catastrophic medical expenses, including maternal and newborn care costs.

The main priority of the government is the further improvement of the healthcare financing system, which is manifested by a significant increase in the allocations allocated to the healthcare sector. The government intends to continue the policy of universal access to medical services for all citizens by gradually increasing the volume of the medical service package, investing more financial resources in various preventive services in order to increase the primary, secondary and tertiary prevention of diseases and reduce the burden of morbidity and mortality of the population; Increase the financial support of the full cycle of maternal and child medical services, which includes: antenatal visits, incl. screening tests for: HIV infection/AIDS, hepatitis B and C and syphilis; treatment of pregnant women suffering from syphilis, antiretroviral and anti-syphilis treatment of mothers; services for children up to 0-18 years; antenatal

Genetic screening (triple test and amniocentesis); childbirth and caesarean section for all women; Screening of newborns and children for hypothyroidism, phenylketonuria, hyperphenylalaninemia and cystic fibrosis; Expanding newborn hearing screening, and other services already covered by universal health care and other government (so-called vertical) health care programs.

Despite the successes achieved, state institutions are still facing many challenges. Establishing appropriate funding and output/indicator-based payment (P4P) system for service providers is actively being considered.

3. Delivery of services

Good health of mothers and newborns requires an efficient healthcare system. Such health care systems play a critical role in promoting health, saving lives, and preventing disability in both the short and long term.

Women need counseling about the risks associated with pregnancy and modern family planning methods. During pregnancy, they should be monitored by a multidisciplinary team and the place of expected delivery should be determined in advance in a facility providing perinatal services of the appropriate level. They should be monitored in the early postnatal period through postnatal care and preferably within 42 days of delivery under the care of a health care provider.

In 2013, the nationwide census of medical institutions and their data placement on the electronic health portal began, followed by the detailed passporting of 1,553 medical institutions, which includes information about the infrastructure, human and administrative resources of these institutions[50] .

Currently, perinatal services are concentrated in 108 specialized or multi-specialist inpatient centers providing obstetric and gynecological services. More than 90% of these institutions are private, however, they usually participate in government programs as service providers, along with state-owned clinics.

Despite the relatively high coverage of antenatal care in Georgia with four complete visits (84.6%), there are different data between the rural and urban population regarding the initiation of the first visit before the first 12 weeks of pregnancy (86% and 93%, respectively) [51] . This may be due to the low level of awareness of the population about the importance of antenatal care. Presumably, this variable should be correlated with mothers' socio-economic status, age and education[52] .

At the same time, practice of prenatal and postnatal care is largely absent in Georgia, and routine gynecological visits (without pregnancy) are rare. Unlike antenatal care, postnatal care services are not separately/targetedly funded by the government. According to the 2010 Reproductive Health Survey, only 23% of women had a postnatal visit, and only 31% of them visited a health facility within a week of giving birth, as recommended by the World Health Organization.[53]

Although the proportion of births in facilities has increased from 92% in 1999 to 99.9% in 2010[54], the total number of births in many maternity facilities is less than 500 per year[55]. It is in the process of implementation for the timely detection, care and timely referral of high-risk pregnant women and newborns at various levels (primary health care, women's counseling center, obstetrics and

appropriate mechanisms for information sharing and feedback between perinatal facilities) because in a number of cases, maternity homes, especially in areas that do not have adequate capacity to manage obstetric and neonatal emergencies (lack of staff, medicines, equipment, blood banks, etc.), Either they do not refer the pregnant woman to a higher level institution, or they do so with a significant delay[56] . An important part of an effective referral system is a well-organized and centrally coordinated transportation system, but it is still underdeveloped, largely because of the mass privatization of transportation services by the private sector, which manages it according to its own priorities.

The start of the process of regionalization of perinatal care is an important step forward in the direction of strengthening the maternal and newborn health care system in the country. The project was implemented by the Ministry of Labor, Health and Social Protection of Georgia with the support of donor organizations. Regionalization of perinatal services involves defining the levels of facilities and their roles and responsibilities to ensure effective patient referral when needed. The pilot project was started in Imereti and Racha-Lechkhumi maternity hospitals in 2015, with the support of the United States government (US Agency for International Development/SUSTAIN). Later, the project was expanded and implemented in Kvemo Kartli and Tbilisi together with the United Nations Development Fund. Since July 2016, the regionalization process continued in Kvemo Kartli, Adjara, Guria, Samegrelo, Mtskheta-Mtianeti and Shida Kartli regions with the support of the United Nations Children's Fund, and in 2017 in Kakheti and Samtskhe-Javakheti with the support of World Vision.

As a continuation of the regionalization process and its additional incentive, in order to improve the quality of care for mothers and newborns, with the support of the United Nations Population Fund, the methodology for improving the quality of perinatal care provided by the World Health Organization was introduced in three pilot maternity hospitals of the Imereti region - near-miss medical cases.) review, which will be gradually implemented in other regions as well.

Although several initiatives have been implemented to provide pregnant women with folic acid and iron-containing preparations, and children aged 6-23 months with nutritional supplements containing microelements, maternal malnutrition as a factor contributing to maternal and newborn morbidity and mortality is still insufficiently recognized. For women, before pregnancy and during pregnancy, a varied and complete nutritional regime is necessary. For example, folic acid supplementation before pregnancy reduces neural stem defects in children and prevents stillbirths [57] . Iodine deficiency causes natural abortion and stillbirth[58] . Maternal anemia, caused by the deficiency of iron and other trace elements in food, is an indirect cause of maternal mortality. It also contributes to fetal growth retardation and low birth weight [59] .

Early and exclusive breastfeeding requires active support at all levels of care. After 1994, the government of Georgia fully supports the breastfeeding program in the country. In order to facilitate the implementation of the International Code of Marketing of Breast Milk Substitutes, in 1999, the Georgian Law "On the Protection and Promotion of Natural Nutrition of Children, the Use of Artificial Foods" was adopted by the joint efforts of the Georgian Ministry of Labor, Health and Social Protection and the Georgian group "Klaritas" of the International Infant Nutrition Association (IBFAN). ", which prohibits the advertising of artificial food products in any form. An exception is supplementary food. Nevertheless, the country needs to implement law enforcement mechanisms and monitor it.[60]

4. Human resources

Human resources planning and management are important elements of sustainable progress in the field of maternal and newborn health care. Overall, Georgia has a surplus of doctors (573.3 doctors per 100,000 population as of 2015), but the country has a persistent shortage of nurses (419 per 100,000 population in 2015). Both indicators are significantly different from the average number of doctors and nurses in the European region[61]. At the same time, the medical staff is unevenly distributed in the regions, in particular, a large part of it is concentrated in the capital or another big city. The same applies to individual medical specialists: obstetricians-gynecologists, anesthesiologists, neonatologists.

With the support of donor organizations (USAID/JSI, UNICEF, UNFPA), in 2015[62], large-scale training of obstetricians-gynecologists, neonatologists, midwives and neonatal nurses was conducted in perinatal services facilities according to evidence-based internationally recognized maternal and newborn guidelines and protocols. Nevertheless, there are still significant gaps in the availability of qualified specialists, largely due to the country's lack of mandatory continuing medical education, which is considered a key component of continuing professional development.[63] Continuing medical education is necessary to acquire and maintain knowledge, attitudes, behavior, and clinical skills; It is also a fundamental factor in maintaining the certificate confirming the right to independent medical activity[64]. However, in Georgia, in the absence of state requirements for recertification, a one-time state certificate remains valid for life. This does not encourage doctors to systematically update their knowledge and skills.

Another important problem is the lack of professional and financial incentives for healthcare specialists, especially in the regions, which leads to high turnover and, ultimately, a shortage of qualified personnel. This creates additional barriers for access to appropriate medical services in these settlements. In addition, in some remote and mountainous areas, the number of births is extremely low (ranging from two to five months), which prevents the accumulation of necessary clinical experience and the maintenance of appropriate skills, as well as reduces the motivation of medical staff to be oriented to international evidence.

5. Quality of service

Ensuring quality of care requires the existence and implementation of specific, evidence-based service standards of care. However, such standards either do not exist or are not used in practice.

The process of developing and updating clinical guidelines is fragmented. Since 2014, a total of 159 national guidelines and protocols have been approved, including 12 for the management of major obstetric and neonatal conditions, but the use of these guidelines and protocols in daily practice by healthcare professionals is quite limited, which significantly affects the quality of services provided.

In Georgia, where the coverage of medical services is high, by improving the performance of the health care system and through regular clinical audits, it is possible to reduce the high rate of adverse maternal and newborn health outcomes. However, promoting the improvement of service quality, against the background of relatively weak supervisory mechanisms in the private sector, is a serious challenge for the Ministry of Labor, Health and Social Protection of Georgia. Although the improvement of the quality of medical services and the development of patient safety mechanisms are part of the requirements of the license/permit and technical regulation of high-risk medical activities, the management of medical institutions has no other type of motivation for the continuous monitoring of the quality of medical services and

For improvement. Quality assurance mechanisms based on internal and external audits are not implemented. There is a need to improve the existing regulatory approaches or introduce new mechanisms, such as accreditation, recertification, etc., which will contribute to the compliance of both medical institutions and the personnel working in them with the standards defined at the national level.

In order to solve the challenges in the field and to reduce the mortality of mothers and newborns, the Ministry of Labor, Health and Social Protection of Georgia established the Maternal and Child Health Coordinating Council, which includes the leading specialists in this field - experts, however, the capacity of the system needs to be further strengthened.

Due to the high caesarean section rate in the country (35-38%) in 2013, the Coordinating Council for Maternal and Child Health promoted the development of a clinical guideline and protocol for caesarean section, which strictly defines the indications for cesarean section. Nevertheless, the rate of surgical deliveries and especially primary caesarean sections is still high in Georgia. It is necessary to implement a proper clinical audit system in order to create an accurate representation of the factors that contribute to the emergence and maintenance of such trends; It is necessary to supervise the performance of caesarean section in Georgia and develop/implement a policy aimed at reducing it. At the same time, it is advisable to prepare interventions to encourage physiological childbirth.

6. Healthcare information system

To monitor progress towards global, national and local goals and targets, the availability and quality of data on maternal and newborn mortality and health needs to be improved to inform decision-making. In order to more effectively monitor national results, the Commission on Information and Accountability on Maternal and Child Health Issues highlighted the need to improve data through the active involvement of international partner organizations, national governments, the public and the civil sector to ensure targeted monitoring, accountability and effectiveness.

Civil registration and demographic statistics systems of 148 countries (CVRS) recent global assessment (period 1980-2012) using the combined index of demographic statistics showed that, according to the classification, Georgia is included in the group of countries that have full access to data on causes of death and operational systems, but still face the old problem Regarding data quality [65] . Despite these limitations, the study confirmed the progress made by the Georgian government in improving the country's civil registration and demographic statistics system; Special attention was paid to the coverage of the registration of deaths, which was achieved as a result of the introduction of institutional and legislative reform since 2009 (such as mandatory notification within five days of death, electronic medical certificates of death, etc.). This was confirmed by the GERAMOS 2014 study, where in 2012, unlike the first RAMOS study in 2006 (GERAMOS 2008)[66], not a single unregistered case of death of women of reproductive age was detected, however, like GERAMOS 2008, this study found 23 maternal deaths From the case, 9 unaccountable cases were identified as cases of death of the mother. Most of them were due to indirect causes of death (when death occurred after 42 days of termination/termination of pregnancy). These reasons were classified in the group of other reasons by official sources.

There are several tried and tested methods to improve the statistical recording of maternal mortality, such as, for example, on medical records of death.

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Attaching the pregnancy questionnaire and relating the woman's other medical histories to the birth histories. The latter, based on the recommendations of the first RAMOS study, has been successfully used since 2008 by the National Statistical Service of Georgia to identify mothers who died within one year of childbirth. However, this method cannot be used in cases of death of women who have not given birth.

Perfection of death registration and determination of the exact causes of death is an important prerequisite to be able to use the data in policy making to make the right decisions. From this point of view, the main problem remains the high share of wrongly determined causes of death, most of which - 50% - come in 2010. According to the 2014 RAMOS study, 21.7% (5/23) of maternal deaths fall into this unspecified category. In connection with the mentioned issue, the Ministry initiated a special program for registering the causes of death; With the participation of regional public health centers, a secondary investigation of incorrectly determined causes of death was conducted through data collection (the so-called "verbal autopsy"); The technical validation and control software was developed by the US Agency for International Development-funded Healthcare Quality Improvement Project and the Ministry of Labor, Health and Social Protection of Georgia. As a result of this, the share of incorrectly determined causes of death decreased to 29% in 2014[67] . Nevertheless, there is a need to further strengthen the human resources and monitoring capacity of the system of civil acts registration and demographic statistics.

In 2013, the Ministry of Labour, Health and Social Protection of Georgia introduced a system of emergency notification of maternal deaths, deaths of children under 5 years of age and stillbirths. Healthcare service providers are obliged (Order of the Minister of Labor, Health and Social Protection of Georgia - 23.07.2013 No. 01-30/N) to call the relevant hotline and inform about the case of death. The information must be transmitted electronically to the Department of Health Protection of the Ministry of Labor, Health and Social Protection of Georgia within 24 hours after the incident. After that, the institutions are obliged to submit a copy of the medical history related to the case within five days to the Health Protection Department of the Ministry of Labor, Health and Social Protection of Georgia.

LLP under the state control of the Ministry of Labor, Health and Social Protection of Georgia - L. Since 1996, the Department of Medical Statistics of the National Center for Disease Control and Public Health named after Sakvarelidze has been collecting monthly and annual data on maternal and child health from all medical institutions. The data of LSI-L. The institutions providing perinatal services provide information on antenatal, intranatal and postnatal care, as well as maternal mortality, early neonatal deaths and stillbirths to the Sakvarelidze National Center for Disease Control and Public Health.

In 2012, SSP-L. The National Center for Disease Control and Public Health named after Sakvarelidze carried out active surveillance of deaths of women of reproductive age (15-49 years). Since 2015, the system also includes under-5 mortality. Notifications are recorded by local public health services, which are responsible for collecting information from local health facilities through an electronic integrated disease surveillance system.

In January 2016, the Ministry of Labor, Health and Social Protection of Georgia, L.S.P. Together with the National Center for Disease Control and Public Health named after Sakvarelidze and with the support of the United Nations Children's Fund, a new electronic registration system - "Pregnant and Newborn Health Surveillance" was launched

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The so-called Implementation of the "Birth Registry". The system contains information on pregnancy, delivery, postpartum period, abortion, as well as maternal deaths, stillbirths and early neonatal deaths. However, an effective and user-oriented health information system that supports data collection, communication and coordination between different levels of service, as well as between service providers and patients, still needs further refinement and development.

2.3. Tasks and priority interventions for improving the health of mothers and newborns in Georgia

Goal 1: By 2030, all women will have universal access to evidence-based prenatal, antenatal, obstetric, neonatal and postpartum care

Priority interventions:

a) Creation of a supportive environment for maternal and newborn health programs

Sustainability of program implementation is possible only if commitment is made at all levels to improve the maternal and newborn health care environment. The complexity of this issue shows that no single ministry, agency or sector has all the necessary resources, competence and authority to implement all interventions to achieve the desired results. Therefore, alongside government efforts to improve maternal and newborn health, a multisectoral approach to collaborative and sustainable partnerships within and outside the health sector, including with the private sector, must be continuously strengthened. Advocacy by members of parliament, state and local governments, and civil society can build decisive political will to make equity a priority and expand maternal and newborn health policies and programs. The Ministry of Labour, Health and Social Protection of Georgia, together with its Maternal and Child Health Coordinating Council, has an important role in identifying the necessary partners and determining their potential role for maternal and newborn health protection purposes. It is also important to advocate for the allocation of budgetary funds by the Ministry at all levels to ensure that other sectors perform health-related functions appropriately to support and facilitate the maternal and newborn health care program. To achieve all of the above, the results of annual reports on maternal and child health should be made public.

All women should have access to high-quality and safe health care services that respect their privacy, privacy and informed consent. At the same time, their cultural values and religious beliefs will be respected. This will help to meet the real health interests of consumers (mothers, newborns and families). The maternal and newborn health care strategy aims to improve the legal framework for the protection of patients' rights to ensure the provision of affordable, acceptable and comprehensive health care services and to provide information to citizens so that they know their rights and how to exercise these rights.

b) reduction of geographical, financial and socio-cultural barriers related to the availability of services for mothers and newborns

Differences between people who are most or least likely to be affected by effective interventions can be eliminated by a universal health program or other

Universal coverage of maternal and newborn health care services within health (so-called vertical) programs, which includes two equally important parameters: ensuring everyone's access to services and protecting them from catastrophic health costs.

The introduction of the universal health care program, together with the increase in public spending on health care, undoubtedly had a positive effect on the increase in the consumption of medical services. However, in order to reduce or completely eliminate the financial, cultural and structural barriers that prevent access to health services, primarily for the most vulnerable populations, a systematic analysis of these barriers is needed to ensure the exchange of relevant information for coordinated action between different sectors and agencies. . This will contribute to the correct determination of priorities in terms of maternal and newborn health care and the distribution of adequate and sustainable resources. Financial incentives for the poor, culturally sensitive programs and other effective activities are key interventions that require additional investment and expansion. Evidence shows that as a result of national or social insurance programs, tax credits, conditional cash transfers and vouchers, the use of antenatal and obstetric services has increased in a relatively short period of time and on a relatively large scale . [68]

To improve access to preventive health services, it is important that a minimum package of maternal and newborn care is available to residents of rural and remote communities to provide them with family planning services, rapid emergency management and timely referral. Primary care providers, if properly trained, can help families strengthen prenatal care and nutrition practices, help prepare the baby for birth, recognize warning signs, and get needed services during pregnancy and after delivery through pre-planned home visits. These measures require training, equipment, as well as travel and transportation support or other incentives in order to maintain the motivation of primary care providers. The practice of home visits for maternal and newborn health has shown significant reductions in preventable morbidity and mortality.[69]

Given the fact that mobile phones are increasingly used by different categories of the population, it is possible to use "mobile health" (mHealth) to provide expanded and improved health services to individuals and entire communities, which will help to strengthen health systems using one-way, two-way and multi-way communications. [70] The most frequently identified one-way way of communication, i.e. the so-called One-touch communication involves serving large audiences with short text messages, especially in underserved and hard-to-reach rural areas, to convey information focused on personal needs, including reminders of scheduled follow-up visits (eg, antenatal and postnatal care visits, etc.). The Minimum Reproductive Health Service Package (MISP), which is a priority set of life-saving measures to be implemented immediately after a humanitarian crisis, will be integrated into the sectoral emergency preparedness and response plans of the Ministry of Labor, Health and Social Protection of Georgia.

c) Enhancing population mobilization and participation to improve access to and demand for maternal and newborn services.

Evidence shows that population mobilization and participation has a positive effect on maternal and newborn health care, especially in rural areas[71] .

The mentioned efforts are aimed at increasing the responsibility of the population for their own health and well-being. Intervention involves identifying problems at the community level, identifying root causes (such as barriers to accessing health services), mobilizing needed resources, demanding rights to health and quality services, and promoting supportive societal norms. As part of population engagement, influential public and religious leaders, civil society organizations, and local decision-makers will significantly contribute to social mobilization and gaining population support to strengthen connections between families and the health system. Such mobilization can create demand for services, reduce barriers to access, and ensure continuity of care for women and newborns, allowing them to fully benefit from the services they use. International practice testifies to the success of similar measures in terms of reducing the use of medical services, referral in case of complications, maternal morbidity, stillbirth and perinatal mortality[72] .

Task 2: By 2030, the quality of medical services for mothers and newborns will be improved, services will be integrated and standardized

a) Strengthening the continuum of care for mothers and newborns by increasing the efficiency of preconception, antenatal, intranatal and postnatal care services and their connecting referral system

High-return, cost-effective interventions, including the inclusion of pre-conception (pre-pregnancy) and post-natal care packages in the basic package of primary care, are of great importance in the service delivery process; Implementation of a stratified model of extended antenatal care by defining different levels of pregnancy patronage to introduce measures for early detection, treatment and timely referral of high-risk pregnancies; Also, elimination of mother-to-child transmission of HIV/AIDS and syphilis; improving the obstetric and neonatal as well as referral system by regionalizing perinatal services across the country, and strengthening the emergency referral system for high-risk mothers and sick children; Also, implementing an electronic health record (EHR) system to improve functional and informational linkages between different levels of care within maternal and newborn health care.

Promoting healthy nutrition before and during pregnancy is an important component of maternal and newborn health and survival, as well as improving the effectiveness of iron, folic acid and other micronutrient supplementation programs by strengthening their supply chain systems and ensuring adherence to established protocols.

At the same time, it should be noted that the support of early-started, exclusive breastfeeding and the use of expressed breast milk are extremely important for the health and survival of newborns. In this sense, effective interventions should be aimed at eliminating barriers to exclusive breastfeeding and promoting baby-friendly hospital initiatives.

b) Improving the quality of maternal and newborn care by implementing process improvement, advocacy, regulatory and reporting mechanisms

Although the number of facilities providing maternal and newborn health services has increased significantly over the past decade, the quality of services in many of them is not uniform. In cooperation with professional associations and other stakeholders, the authorities should regularly update national guidelines for interventions in the context of maternal and newborn care.

Policies, guidelines, norms and standards in line with globally recognized, evidence-based guidelines and national strategies, and ensure their implementation. Standardized quality indicators should be introduced at the national level to monitor the quality of key interventions, including the management of maternal and neonatal life-threatening complications. A system of regular audit of maternal and perinatal/neonatal deaths, near-deaths and also discussions among health care personnel to identify causes and challenges that can be avoided should be introduced. This type of audit has the potential to reduce preventable deaths. Therefore, in an optimal situation, it is desirable to carry out these measures at least at the level of clinics. The fundamental principle of conducting this audit is a confidential, secure environment, where cases are discussed only anonymously, resulting in a more complete picture of the development of events, which, in turn, allows for the development of useful recommendations aimed at improving the quality of service. In this regard, some changes in the legislation should be made in order to improve the system of review of maternal and newborn deaths, which is important for conducting a full-fledged confidential audit of maternal mortality and introduce an approach to maternal and newborn mortality cases instead of the current "personal" approach.

systemic

c) raising the level of competence of service providers and providing supportive policies, budgets and regulations to solve the problems of the qualification level of the necessary personnel, their distribution, retention and motivation

Interventions with high efficiency and impact are possible only if the medical institutions are properly staffed, the medical staff of the clinic is qualified, has the skills to correctly assess patients, as well as to save lives and provide high-quality medical services. Therefore, it is essential to identify existing human resource gaps, develop policies, budgets and regulations aimed at retaining and motivating medical personnel in the field. Improving the competence of health care providers includes pre-training and on-the-job training based on the latest national guidelines for maternal and newborn health care. In order to solve problems related to human resources, it is also very important to introduce a mandatory system of continuous medical education and recertification of medical personnel. The continuing medical education course should be based on an evidence-based, unified educational plan and innovative teaching methods so that service providers who have the appropriate certificate, trained and tested according to the appropriate standard, so that they can provide high-quality, modern and patient-oriented services.

d) Improvement of financing mechanisms for maternal and newborn healthcare services

Increasing the motivation of maternal and newborn health care providers is important to determine the quality of services. Different incentive mechanisms can be used, including the introduction of a special form of incentive based on output/result-based payment system (P4P), or combined systems of different payment methods, which are already being introduced in several countries as a means of improving service quality.

e) Strengthening the health management information system and research capabilities

The country should invest more in regular measurement and collection of data on service outcomes, coverage and quality. In addition to standardized data sources, indicators and data collection intervals for comparison with global data, it is an important component of program effectiveness.

New opportunities and challenges for health systems require more investment in demand-oriented research (household population-level surveys, operational research) and analysis; Also, in state-led, evidence-based processes aimed at making sound and effective decisions by policy makers, program developers and medical staff.

Priority interventions:

The prospects for both maternal and child health improve when all women and families, including girls, have the right knowledge about how to seek help at the right time and in the right place, even more so when their male partners and communities support them. in the implementation of healthy behavior. Healthy behaviors include choices about whether women want to become pregnant and when; Conceiving without infections and with good nutritional status; Also, whether she will maintain a healthy diet during pregnancy, delivery and after delivery.

Supporting men's involvement in improving reproductive health and maternal and child health will contribute to the achievement of the set goal. Culturally tailored information and messages can be shared through many different channels, such as mass media, social media, personal counseling, mobile and other innovative technologies.

<http://www.matsne.gov.ge>

Advocacy and social mobilization are additional beneficial elements, which help to change and increase the inhibiting traditional beliefs demand for high-quality services.

Advocacy plays an important role for all categories of stakeholders In raising awareness and motivation, mobilizing resources and services and behavior To accelerate the implementation of change-related communication programs.

Social mobilization includes capacity building and cross-sector the process of cooperation both at the state and public level, as well as at the civil level Society, non-governmental organizations, religious groups, media and private Sector engagement, which will be directed towards behavior change activities to support. especially the establishment of partnerships between the public and private sectors Promotes multimedia advocacy campaigns in the private sector Communication platforms, television, radio, social media and mobile Using health technologies.

The table below shows the targets and indicators mentioned Indicators will be used during the implementation of interventions To evaluate the results.

Objectives and related outcome indicators	Basic 2015*	Target 2030
Mother's sister Newborn health		
Maternal mortality rate (per 100,000 live births) (Sustainable Development Goal 3.1.1)	32.0	12.0
Neonatal mortality rate (/1000 live births) (Sustainable Development Goal 3.2.2)	6.1	5.0
Stillbirth rate (per 1000 births)	9.7	6.8
All women will have universal access to and use of evidence-based prenatal, antenatal, Obstetric, neonatal and postpartum services		
Percentage of women who received Preconception Care N/A 100%		
Percentage of women aged 15-49 who scored 4 or more Antenatal visit	86%	100%
Percentage of women who have given birth Follow-up visit to the medical facility 6 weeks after delivery within	N/A	100%
Percentage of newborns who received a pediatrician Consultation during the first week after discharge from the maternity hospital	N/A	100%
The quality of medical services for newborns will be improved, services for mothers and sisters will be integrated standardization		
Perinatal mortality rate (per 1000 births)	13.4	8.0
Early neonatal mortality rate: 0-6 (/1000 on a live birth)	3.7	3.0
Late neonatal mortality rate: 7-28 (/1000 on a live birth)	2.5	1.7
The percentage of live births (in the clinic) at birth is low by weight (<2,500 grams)	6%	5%

Percentage of cesarean deliveries	41%	27%
The population's awareness and knowledge about healthy behavior, high-quality medical service standards and patients' rights when using these services will be significantly improved.		
Proportion of women of reproductive age who will be able to conceive and recognize the danger signs of melogin	N/A	>90%
Proportion of women of reproductive age who will be able to recognize danger signs in newborns	N/A	>90%

* Latest available data of 2015

3. Family planning

Family planning is one of the main prerequisites for improving the health of mothers and children. Through family planning, couples can avoid unwanted pregnancies and abortions, high-risk pregnancies at very young or very late ages, and maintain recommended intervals between pregnancies. Unintended pregnancies and subsequent births are well-known risk factors for maternal health and are also associated with health and developmental risks for children born. Adequate child spacing reduces maternal and child morbidity and mortality. Abortion is not the preferred method of preventing unwanted births because, among other reasons, it carries risks for a woman's health and subsequent fertility. Healthy reproductive behavior has a positive impact on future generations. Effective family planning reduces the number of unwanted pregnancies and unsafe abortions, makes it possible to prevent pregnancy-related mortality and morbidity, reduces the frequency of sexually transmitted infections, including HIV and AIDS, and protects the health of adolescents. In addition, family planning is one of the most cost-effective ways to improve the health and well-being of women, men, youth, and society as a whole. For all these reasons, investing in modern family planning methods is one of the most effective interventions to improve maternal, newborn, and child health.

3.1. Development of family planning and its current situation in Georgia

Despite national legislation aimed at protecting people's reproductive rights and the right to access reproductive health services, and despite significant resources and technical facilities being invested in the health sector, access to and use of family planning services in Georgia is still quite limited. Also, family planning consultations and services at primary health care level are limited.

Reproductive health studies conducted in Georgia in 1999 and 2010 showed some progress in this direction, however, family planning still remains a significant challenge, as the number of unwanted pregnancies is still high, the demand for modern family planning methods is not adequately met, and artificial insemination is common. Abortion, which is still the main method of family planning, and its frequency can be reduced only by increasing the use of effective family planning methods. Since there is virtually no representative data since the last reproductive health survey in 2010, the recent changes in family planning and its status as of 2016 are largely unknown.

It should be noted here that the recent increase in the birth rate in Georgia is correlated with the increasing prevalence of contraception. In particular, the level of contraception use has increased signific

Moreover, the number of abortions and at the same time, the birth rate increased. According to the data of the reproductive health studies conducted in Georgia, it is expected that the prevalence of contraception will further increase by 15-20 percent without endangering the increase in the birth rate and, at the same time, the rate of induced abortion will continue to decrease. The latter will help reduce secondary infertility, which is associated with high rates of abortion and associated pelvic infections (37% of women aged 35-44 do not use contraception due to female infertility [73]). The real choice facing the country is family planning over abortion.

1. Unwanted pregnancy

According to the 2010 Reproductive Health Survey, only 41% of all pregnancies in Georgia in 1999 were wanted. This percentage increased to 48% in 2005 and 63% in 2010. In 2010, the rate of unwanted pregnancies was 26%, and 11% of unwanted pregnancies (they wanted to get pregnant later). Most of the unplanned pregnancies occurred among relatively older women; 39% of pregnancies among women aged 30-34 and 54% of pregnancies among women aged 35-44 were unplanned. A large number of unplanned pregnancies did not end in a live birth (94.4% ended in artificial abortion, natural abortion or stillbirth). This means that family planning not only reduces the birth rate, but also reduces the number of abortions.

2. Use of modern methods of contraception

Although the rate of overall contraceptive use has increased from 41% (in 1999) to 54% (in 2010), Georgia still has the lowest rate of overall contraceptive use among Eastern European countries[74] . In Georgia, the use of traditional methods (calendar method and interrupted intercourse) remained almost unchanged. About one-third of consumers, or 18.5% of married couples, still use these unsafe methods. This rate is higher than the average European rate (13.9%) and especially higher than the Western European rate (3.2%). Of all married women in Georgia in 2010, only 35% used modern methods of family planning, mainly condoms or intrauterine devices. The use of oral contraceptives (4%) was significantly lower than the overall European rate (21.4%).

3. Knowledge about contraception

Knowledge of modern methods of contraception among women of reproductive age was still low in 2010 and was not obtained from reliable sources. Condoms and intrauterine devices were fairly well known, but 19% did not know about oral contraception, and 50% did not know how to use this method. Only 39% of women knew about female sterilization, while male sterilization was almost completely unknown (4% knew about it). For only 17% of women, the doctor was the most important source of information. This may have been the reason for the lack of knowledge about the advantages and disadvantages of different methods of contraception.

4. Unmet need for family planning

According to a 2010 reproductive survey, 12.3% of married couples did not use any method of family planning. Additionally, 18.2% of at-risk and married women used an unsafe method of contraception. Together, this accounts for 30.5% of the unmet need for family planning contraception, which is very high compared to European standards, especially in rural areas where this figure reaches 40%.

5. Artificial abortion

Abortion is still a widely used method of birth control in Georgia. Until 2000, Georgia had the highest documented rate of artificial abortion in the world - 3.7! According to the 2010 Reproductive Health Survey, this figure has decreased to 1.6. According to the latest data, the abortion rate in Georgia (36.7 abortions per 1,000 women aged 15-44 in 2015) was higher than the similar rate in Southern Europe annual rate (26) and twice as high as in Northwestern Europe (18) [75]. Nevertheless, the data presented must be treated with caution, as it is highly likely that a certain number of induced abortions are recorded as "natural abortions" or as "stillbirths", since the stillbirth rate is approximately twice the international average. In conclusion, it can be said that the abortion rate in Georgia is still high, which clearly indicates the low availability and use of family planning services.

6. Family planning services and service quality

Reducing the incidence of induced abortion is one of the most important priorities of the national health system in relation to fertility and reproductive health. Over the past decade, much has been accomplished through the introduction of free, modern family planning methods and increased access to them; With the help of the United Nations Access Fund and the US Agency for International Development (through the John Snow Institute), primary care physicians were trained to provide family planning services and the health system was provided with free modern contraceptives. In 2014, the Ministry prepared a package of regulatory mechanisms for abortion, which includes the following documents: "On approval of the rules for the implementation of artificial termination of pregnancy" Order of the Minister of Labor, Health and Social Protection of Georgia dated October 7, 2014 No. 01-74/N; "Safe termination of pregnancy" - the state standard (protocol) for the management of clinical conditions and "abortion procedure" - the patient's guide on approval" of the Minister of Labor, Health and Social Protection of Georgia dated July 28, 2014

Order N01-182/o). This has led to the inclusion of family planning services in the purview of primary health care providers, particularly family physicians.

There is international consensus that the vast majority of contraceptive methods can be safely distributed or prescribed at the primary care level, and only a few methods require (based on oral history) screening for some very rare medical contraindications[76]. Since the majority of family planning activities do not require specific knowledge and skills, integrating them into primary health care will avoid unnecessary costs associated with training narrow specialists and referring patients to them. Despite this evidence, family planning counseling for primary care physicians has not yet been fully integrated at the primary care level and is still largely concentrated in the specialty of obstetrics and gynecology, which has traditionally shown little interest in actively promoting the use of modern methods of contraception. It should be noted here that there is a separate specialty in the country - reproduction, within the framework of which specialists are already being trained with a new residency program. Reproductology is focused on such topics of reproductive health as family planning, safe contraception, etc.

According to the recommendation of all international expert organizations, family planning services should be integrated into primary health care, which is also one of the main goals of this strategy. If necessary, a referral to a specialist should be provided. Providers of primary health care and maternal and child health services must have: the knowledge, skills and resources necessary to provide high quality reproductive health services[77]; This depends on the institution qualified

provision of medical personnel, compliance with quality standards, availability of medical equipment and supplies in institutions; The appropriateness of referrals and the high level of customer satisfaction. The experience of the past years has shown that the successful implementation of the planned changes in the health care system requires the introduction of various types of incentives and regulation mechanisms.

Because the majority of health care providers in Georgia are private sector, regulatory and management tools are mainly limited to the creation and enforcement of quality standards, performance-based pay, and selective contracting aimed at improving service quality and delivery efficiency. The updated system of medical facility licensing/permits/technical regulations for high-risk ambulatory medical activities and doctors' certification in Georgia will allow the Ministry of Labor, Health and Social Protection of Georgia to provide reproductive health and family planning services in accordance with internationally recognized standards.

Overall, there is significant work to be done to implement the Program of Action recommendations that emerged from the International Conference on Population and Development (1994) and to achieve global target 3.7 of the Sustainable Development Goals to make family planning universally available, including through the primary health care system. by means of

7. Distribution of financial resources

Currently, no state budget funds are allocated for family planning counseling or service provision. These services are not included in the service package for users of state programs or private insurance. Also, contraceptives are not included in the list of medicines of the state universal health program.

8. Safety of contraception

The United Nations Population Fund and the US Agency for International Development were the only providers of free, modern contraception for those who needed it most, although the practice ended in 2015. Currently, contraceptives are not provided to the population through public sector health programs. They are available on the private market. Due to recent regulatory changes, oral contraceptives have fallen under the category of "emergency contraception" (over-the-counter medications); however, the market price of listed oral contraceptives is quite high, so they are not readily available for many women in need, especially in rural („areas.

The 2010 Reproductive Health Research Report concludes that "reducing the unmet need for modern contraceptives in Georgia will require significant expansion of programmatic and financial assistance" and states that "a national reproductive health strategy should provide free or low-cost contraceptives, provide women with information on what methods and services are available to also disseminate accurate information to help women overcome misconceptions about modern contraceptives" (p. 173). Although there is a clear need for new evidence from the fourth round of the Reproductive Health Study or a similar study), these recommendations are as relevant in 2017 as they were in 2010. Efforts need to be intensified to achieve the Sustainable Development Goals set for 2030 (Target 3.7), which include universal access to sexual and reproductive health, including family planning, information and education services, and the integration of reproductive health into national strategies and programs.

The objectives of this strategy will only be met if the entire population is provided with affordable, quality reproductive health and family planning services and information on healthy behaviors related to sexual and reproductive health. The government intends to implement measures to ensure that the population has access to affordable and quality reproductive health and family planning services at each level of the health system. Such complex, integrated, evidence-based clinical and public health approaches to population reproductive health and family planning needs enable both the public and private sectors to meet these needs much more effectively.

3.2. Priority interventions to improve family planning services in Georgia

Objective 4: By 2030, family planning services will be readily available to all those in need

a)_ Strengthening the role of the state in the field of family planning in order to effectively manage, coordinate and integrate services at different levels

Priority interventions:_____

- Family planning is considered one of the priority directions in the context of health sector reform. It includes initiatives aimed at strengthening the primary health care system so that these services meet relevant medical needs across the lifespan. Also, equality of access to information and services will be ensured.
- Family planning services and modern methods of contraception will be included in the basic package of the universal health program for the target group of beneficiaries.
- The Maternal and Child Health Coordinating Council under the Ministry of Labour, Health and Social Protection of Georgia will be strengthened to develop evidence-based policy/strategy and to act as a planning steward in Georgia. The Council will lead/initiate the development of policies, service standards and guidelines/protocols; coordinate activities with other departments/partners from the Ministry and healthcare providers; will advocate and mobilize the resources of donor organizations; In addition, it provides a cross-sectoral approach to family planning strategy implementation.
- As necessary, the Maternal and Child Health Coordinating Council will revise the current legislation in order to create the legal framework necessary for the effective implementation of the given strategy and the corresponding three-year action plan, which will be based on the principle of human rights protection, gender equality and continuity of reproductive health throughout life.

b) Providing reorientation of family planning services

Priority interventions:_____

- Primary care family physicians who have successfully completed a general family planning course will provide consumers with a full range of family planning methods (except for a few methods specifically indicated) based on the primary care family planning service protocol, including emergency contraception methods.

In the context of continuous medical services, in order to provide customer-oriented services and support a team approach, referral mechanisms and feedback will be strengthened with the following measures:

- implementation of appropriate referrals at all levels of the healthcare system;
- Improving system capabilities in terms of user referrals between different levels of medical services and establishing proper connections between levels to ensure the effectiveness of said referrals;
- Effective coordination and close cooperation between the state and private sector, in order to reduce the financial barriers facing the population, especially for the needy population.
- Integration of family planning with antenatal and postnatal counseling will be ensured.

Task 5: By 2030, the quality of family planning services will meet international standards

a) Improving the competence of healthcare service providers

Priority interventions: _____

- The training/educational course for family doctors will be updated, as well as a training program for general practice nurses will be established in order to make the primary health care link more qualified, which is necessary to provide high-quality reproductive health and family planning services, in accordance with reproductive health/family planning protocols and standard operating procedures.
- The continuous medical education system should ensure the maintenance of proper qualifications and competence of specialists in the currently existing, largely privatized healthcare system of Georgia.
- The Ministry of Labor, Health and Social Protection of Georgia ensures the improvement of the competence of family planning service providers through the following measures: (a) medical, including continuing medical education, through institutionalization; (b) by integrating high-level human resource standards and requirements into the selective contracting model.
- New and emerging technologies, including e-health and online training courses, will be introduced and supported in the delivery of family planning services across the country.

Appropriate regulations, including performance-based remuneration schemes, will be developed and implemented to incentivize service delivery personnel.

b) Promotion of education of population and patients about family planning

Priority interventions: _____

- The state will increase the resources allocated to preventive measures and promotion of healthy lifestyle, ensuring that information on reproductive health/family planning is included in them.
- Ministry of Labor, Health and Social Protection of Georgia under its control

Subordinated LLC" Together with the National Center for Disease Control and Public Health named after Sakvarelidze, will establish the priorities of communication about reproductive health and promotion of a healthy lifestyle. Priority topics for healthy lifestyle promotion will be determined based on epidemiological information and available data on coverage with basic services. These data will be collected from basic health information systems and periodic population surveys.

- As part of the awareness raising strategy in the country, multimedia campaigns will be conducted to combat existing and newly emerging myths and misconceptions about contraception; Measures will be launched to provide practical information about family planning services, which will help the population to make an informed choice.
- The Ministry of Labor, Health and Social Protection of Georgia and the LLP under the state control of the Ministry "L. Sakvarelidze National Center for Disease Control and Public Health" will cooperate with the media and the non-governmental sector, with the aim of spreading knowledge about family planning to the population and specific target groups.
- The Ministry of Labor, Health and Social Protection of Georgia and the LLP under the state control of the Ministry "L. Sakvarelidze National Center for Disease Control and Public Health" will cooperate with the Ministry of Education and Science of Georgia to develop a comprehensive school curriculum on healthy lifestyle, including reproductive health issues. Age-appropriate information on family planning issues will be included in the training.
- LLP under the state control of the Ministry of Labor, Health and Social Protection of Georgia The relevant departments of the "Sakvarelidze National Center for Disease Control and Public Health" will also assess the special needs of men, adolescents and other population groups with regard to reproductive health information.

c) Implementation and strengthening of regulatory and supervisory mechanisms necessary for the provision of high-quality family planning services

Priority interventions: _____

- The Ministry of Labor, Health and Social Protection of Georgia will support the development/adoption of guidelines and protocols for reproductive health and family planning services.
- Modern methods of family planning (intrauterine means oral and contraceptives) will be made available free of charge to targeted social assistance recipients and other vulnerable groups of the population (women under 21 and/or students and HIV-positive women), provided that contraception is counseled and prescribed by primary care staff or, if necessary, by a specialist after referral to a higher level institution by
- Family planning service packages will be developed, which include the requirements for the necessary equipment, methods, infrastructure and qualified medical personnel at the level of both primary care and specialized services; Appropriate regulatory mechanisms/standards will also be developed.
- It will be developed in cooperation with the private sector for each level of service

Minimum data collection tools for family planning service quality and to monitor utilization; They will be integrated into the national health management in the information system; Data will be used based on evidence To make decisions about the quality assurance of family planning services, as well as Service Providers Selective Contracting Terms of Compliance and Service for evaluation purposes.

· Operational and/or population-based studies (multi-indicator cluster research, reproductive health research), the purpose of which will be further improvement of service delivery quality; administrative or policy proposing solutions in case of detection of limitations; This will happen at all levels Along with the development of tools and channels for the dissemination of knowledge and information.

The table below will be used Indicators of results and targets. The mentioned indicators to evaluate the final results during the implementation period.

Objectives and indicators of results	Basic 2015*	Target 2030
Family planning services will be readily available to all who need them; Quality of family planning services will meet international standards		
Percentage share of unplanned pregnancies	36%	15%
Proportion of women of reproductive age (15-49 years), whose family planning The need was met by modern methods.[78]	34.8%	85%
The percentage of women who are related to family planning The information was obtained from a professional source	17%	60%
In women aged 15-49, the rate of induced abortion per 1000 women of the corresponding age	36.7	25

* Latest available data of 2015

4. Sexual and reproductive health of youth[79].

4.1. Sexual and reproductive health of young people in Georgia condition

Adolescent girls represent a risk group for maternal and newborn morbidity in terms of mortality. Pregnancy during adolescence is very often unplanned And it is undesirable, because girls and boys are not yet ready to start a family. Therefore, starting a family at this age damages their future life prospects. As a result, 20 The majority of girls' pregnancies in European countries are usually completed by the age of one year with artificial abortion. In addition, young people are sexually transmitted At increased risk of infections and HIV/AIDS because they are often unprepared to protect themselves from them. Because of this, the sexual and reproductive health of young people In recent decades, various (international) organizations increasingly attracts more attention.

Sexual and reproductive health of young people in Georgia is quite poor It is a neglected issue. There are very limited studies that have been conducted on sex Relationships, contraception and STI/HIV/AIDS prevention among young people To study knowledge, attitude towards them and behavior. Conducted in 2013-14 The national survey of young people only covered certain reproductive issues in relation to health [80] . The most surprising result of this relatively old study It represents that the sexual behavior of young people is a "taboo" issue, his It is almost impossible to discuss openly and this issue is characterized by extreme "double standard", in the sense that the sexual behavior of boys and young men is acceptable and

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is even encouraged, and sexual behavior of girls and young women is completely unacceptable. The study concluded that 25% and 40% of unmarried girls and young women (in the age group of 15-24) had one or more sexual relations[81] . The mentioned study also showed that the knowledge of young people about sexual and reproductive health and preventive measures is very poor. This is mainly a result of the general lack of available information sources. Also, teaching about sexual and reproductive health in schools is insufficient. However, with the help of the UNFPA Georgia office, the reproductive health component is now included in the national school curriculum. The country also lacks specific youth-friendly sexual and reproductive health services that exist in various European countries. An attempt to create such a service a few years ago was unsuccessful due to various reasons, including infrastructure privatization.

Among many different issues, teenage childbirth is important. According to reliable statistical data[82], the birth rate among girls aged 15-19 increased from 39.9 (for every 1000 women aged 15-19 in 2000) to 48.6 (in 2015). The latter indicates that by the time they reach the age of 20, about a quarter of young women have already given birth. The figure of 48.6 is very high by European standards. In almost all Western European countries, it ranges from 5 to 10. Even in Southern European countries, the mentioned rate is much lower: in Greece and Spain – 10, and in Italy – 7. In most European countries, the abortion rate in this age group is approximately the same or higher than the birth rate[83] . In Georgia, on the contrary, the registered rate of abortion is only a quarter of births in this age group (13 per 1000)[84] .

Due to the fact that the sexual and reproductive health of young people in Georgia is a very sensitive and taboo issue, it is necessary to facilitate the creation of the moral space necessary for the improvement of the situation in this area.

It is important that future studies be conducted using innovative and in-depth methodology to obtain real results. The ratio of early and unofficial marriages, the sexual behavior of unmarried young people (especially girls), the use of contraception and artificial abortion in this group need urgent study.

Support from the government of Georgia is especially important in the process of further introduction of school education about healthy lifestyle that has already started [85] .

An assessment of the possibilities of creating special, youth-friendly sexual and reproductive health services showed that the conditions in the country are still unfavorable[86] . It should recommend the development of youth-friendly sexual and reproductive health services as a primary health care function. At the same time, it should become the starting point and main provider of family planning services.

It would be very helpful if the country shares the experience of several other similar countries that have successfully introduced healthy lifestyle education in schools and/or youth-friendly sexual and reproductive health services.

4.2. Approaches and priority interventions to improve the sexual and reproductive health of young people

Task 6: By 2030, age-appropriate education on sexual and reproductive health issues will be fully accessible to young people;

Machine-generated title: Strengthening the role of the Georgian government in improving the sexual and reproductive health of young people

Priority interventions:

- The Maternal and Child Health Coordinating Council will review all national documents on youth sexual and reproductive health (including prevention of mother-to-child transmission of HIV/AIDS, elimination of congenital syphilis), voluntary counseling and testing, antenatal medical services, emergency obstetrics and etc. to meet needs;
- Government will encourage and, where possible, support initiatives to conduct high-quality research on adolescent sexual and reproductive health.
- The Ministry of Labor, Health and Social Protection of Georgia and the LLP under the state control of the Ministry "L. Sakvarelidze National Center for Disease Control and Public Health" will cooperate with the Ministry of Education and Science of Georgia in order to further refine, pilot and implement the healthy lifestyle school curriculum. All the educational materials needed for teaching a healthy way of life will be developed in accordance with the standard and will be integrated into the school curriculum so that by 2030 all schools (and also vocational schools) will implement the mentioned program.
- The Ministry of Labor, Health and Social Protection of Georgia and the LLP under the state control of the Ministry "L. Sakvarelidze National Center for Disease Control and Public Health" will cooperate with the Ministry of Education and Science of Georgia to raise the qualifications of school doctors by supporting their teaching in sexual and reproductive health issues according to the accredited training program in the field.

The Ministry of Labor, Health and Social Protection of Georgia will create a favorable environment for the inclusion of youth-friendly sexual and reproductive health services in the primary health care system.

b) Promotion of education of young people about sexual and reproductive health issues

Priority interventions:

LLP "L." under the state control of the Ministry of Labor, Health and Social Protection Sakvarelidze National Center for Disease Control and Public Health will prioritize and in cooperation with other interested parties will start information campaigns for young people on various issues of sexual and reproductive health using new (social) mass media, telecommunications (mobile applications) and interactive web platforms.

- Through cross-sectoral collaboration, educational activities will be developed and implemented for civil society and parents on the issues that should be communicated to their children in the field of sexual and reproductive health.

Goal 7: By 2030, young people will have full access to sexual and reproductive health services

a) sexual and reproductive health services for youth

Delivering in a youth-friendly manner

Priority interventions: _____

· Special function in primary health care - for young people
Providing Benevolent Sexual and Reproductive Health Services –
Integration and development of young people (especially peers
(instructors) involvement.

· As a pilot project, youth sexuality in one or two large higher education institutions
and the possibility of creating a reproductive health information and service center
Study and assessment of the ability of the center to meet the existing needs.

· Informing young people about sexual and reproductive health services
on related issues in schools, youth clubs, etc. Activation of the hotline –
Their work is more appropriate in cities where adults can access information
Sourcing and consultation.

The table below will be used Indicators of results and targets. The mentioned indicators
to evaluate the final results during the implementation period.

Objectives and related outcome indicators	Basic 2015*	Target 2030
Young people will have age-appropriate education on sexual reproductive health issues		
Percentage of adults who have attended school Teaching about a healthy way of life (including sexual and on reproductive health)	0%	90%
Young people will have full access to sexual reproductive health services		
Percentage of 15-24 year olds who know that Sexual and reproductive health services receive primary care	Minors can	90%
Adolescent birth rate - per 1000 women aged 15-19	48.6	< 20

* Latest available data of 2015

5. Implementation and management mechanisms

For the successful implementation of this strategy, the Ministry of Labor, Health and
The Ministry of Social Protection will not only supervise its implementation,
but also by advocacy, coordination, cooperation with other partners and
Through negotiation, he will try to fulfill the tasks that are not his direct responsibility
responsibility.

The Ministry of Labor, Health and Social Protection of Georgia is responsible and
Accountable to supervise the health of mothers and newborns
Promotion of the strategy of action and effective implementation of the governance of the following
Through functions: (1) from local and international stakeholders
To ensure the achievement of targets, to manage, coordinate and implement implementation
Monitoring (2) Mobilization of resources, monitoring and their effective use
provision; (3) development and adoption of enabling policies, laws and regulations;
(4) Program and service delivery guidelines/protocols and standards
development; and (5) the core provided by the strategy and related action plan
Implementation of interventions and monitoring of their performance.

The current strategy will be implemented with a broad, multidisciplinary approach to stakeholders

My group comprising relevant ministries and agencies, partners, civil society, professional associations and the private sector. Therefore, in practical terms, implementation requires the use of multisectoral and decentralized approaches in coordinating and managing national efforts.

Implementation and development partners will act under the auspices of the Maternal and Child Health Coordinating Council. The said council will perform an advisory and advisory function to the Ministry and other stakeholders and will facilitate the effective implementation of the Maternal and Newborn Health Strategy, providing a forum for stakeholders to share information and technical innovations. The Maternal and Child Health Coordinating Council will attract representatives from government, civil society/NGOs, development partners and the private sector.

In order to avoid duplication of measures, to strengthen the effective use of resources, to monitor the obtained results and to promote the sharing of knowledge, a close and active coordination framework is needed with the Ministry of Labor, Health and Social Protection of Georgia and the public law legal entities subject to its state control: (a) LSI - State Regulation of Medical Activities An agency that should provide an assessment of the conditions and standards necessary for the production of family planning and reproductive health services; quality, safety and effectiveness of contraceptives by regulating their production, import, distribution and use; (b) Social Services Agency, which should ensure the provision of necessary services in the pre-conception, antenatal, perinatal and postnatal periods through state programs; (c) LSI – L. Sakvarelidze National Center for Disease Control and Public Health, which should provide communication about family planning and reproductive health and promote healthy lifestyles; Providing basic statistical information on women of reproductive age, which is very important for monitoring and evaluating the implementation of the strategy.

For the implementation of the strategy, the Ministry of Labor, Health and Social Protection of Georgia will actively coordinate activities with the administration of the Government of Georgia and the Parliament of Georgia. Profile ministries and agencies will also contribute to the achievement of results in accordance with their mandates. The Ministry of Labor, Health and Social Protection of Georgia will assist and provide technical support to ministries involved in the implementation of the strategy so that they consider maternal and newborn health care, family planning and reproductive health of young people among their core functions.

A concerted effort should be made to reflect the actions of the Action Plan in sector plans and budget lines to allocate resources to the relevant sectors to promote maternal and newborn health, family planning and youth reproductive health. The Ministry of Labour, Health and Social Protection of Georgia will work with partners to provide technical assistance to local self-government bodies to adopt and implement the outcomes and activities of the Maternal and Newborn Health Promotion Strategy at the local level.

UN agencies and other donor organizations are also seen as important parties to the successful implementation of the strategy, contributing to the process with the necessary financial resources and technical expertise. Civil society organizations will also complement the public sector by providing services at the institutional level, mobilizing resources and acting as a watchdog to ensure social accountability and responsibility.

Likewise, partnerships with the media, religious leaders are needed to achieve success

mach with the private sector. Given that more than 90% of healthcare providers in Georgia are privately owned, cooperation with the latter will be extremely important to maintain national standards of healthcare. In addition, the private sector should be particularly involved in the provision of medicines and family planning methods. The strategy includes strengthening and expanding the capacity of private providers to effectively engage in public-private partnerships and deliver an essential package of maternal and newborn health and family planning interventions.

6. Risk assessment

During the development of the maternal and newborn health promotion strategy, the following key assumptions were made regarding its success:

- Political, social and economic stability.
- Intention of public, private and civil sectors to make positive changes to adopt new policies and institutionalize successful initiatives that serve to improve maternal and newborn health and family planning.
- A proper perception and understanding of the importance of scientific research and data for decision-making and supporting policy by people working in this field.
- Continuity of employment/retention of qualified medical personnel and availability of financial resources.

In addition, certain risks have been identified that may affect the implementation process, namely:

- Changes in the economic and political situation in the country.
- Lack of dedicated financial and human resources and reliance on donors.
- Loss of motivation and appropriate attitude by decision-makers, change of decision-makers.
- Resistance to change, slow changes in behavior and attitudes in providers, consumers and society.
- High cost of awareness programs, especially those that use informational materials.

7. Monitoring and evaluation

The success of this strategy depends on the regular monitoring of indicators to measure the progress and results of the interventions. The Ministry of Labor, Health and Social Protection of Georgia will monitor and evaluate the mentioned strategy, both through regular control and periodic studies.

All partners involved in the implementation should collect information on relevant indicators of the work performed by them. The information should be presented in a graphic and format that is standardized for uniformity in data collection, analysis and management. Monitoring data includes more detailed indicators for measuring process and intermediate results (). Population studies that will be conducted during the implementation period of the strategy (e.g. the Reproductive Health Survey) include the interim and final outcome measures.

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Indicators (where possible) have an important strategy question arises that cannot be answered by monitoring and research, operational research can be conducted. The results of current and planned national surveys of the National Statistical Service of Georgia (Sakstat) will also be used to measure some indicators of intermediate results.

Data from these sources will be fed back to the relevant levels – both at the medical facilities and at the national level, to be used in regular reports, quality assurance and planning activities. system and will pay special attention to improving its performance. (e.g. the next work plan in preparation).

Evaluation is an important component of any program implementation and management. If necessary, the strategy will be evaluated and revised as follows:

1. Annual review of indicators/intermediate results: The Ministry of Labor, Health and Social Protection of Georgia will organize a joint meeting to assess the progress of the implementation of the strategy against the set targets and to agree on the priorities for the next period. Such meetings will play a key role in the evaluation of the implementation of the plan and the intermediate and final results, as well as in the preparation of recommendations for the next annual work or long-term strategic planning.
2. Mid-term review of the strategy: the strategy will be evaluated in the middle period of its implementation (2017-2030), in 2025, the results and recommendations will be used to make changes in the interventions, and if necessary, to revise the strategy.
3. Final evaluation: will be done before starting the preparations for the next strategy to take into account the lessons learned.

The mentioned steps will be the basis for monitoring and evaluation of the 2017-2030 National Strategy for the Promotion of Maternal and Newborn Health to ensure the compliance of the implemented efforts with the plan and the compliance of the achieved results with the set targets.

8. 2017-2019 Action Plan for the Implementation of the National Strategy for the Promotion of Health Protection of Mothers and Newborns of Georgia

8.1. Introduction

In order to significantly improve the health of mothers and newborns in Georgia, a long-term national strategy (2017-2030) and a short-term action plan for its implementation for 2017-2019 were developed. Since 60% of under-five deaths are neonatal, the strategy is largely aimed at improving the health of newborns. Maternal and newborn health are significantly affected by the provision of quality family planning services, as well as the sexual and reproductive health of young people. That is why the mentioned directions were reflected in the strategic document and the action plan.

The action plan outlines the short-term goals, interventions, targets and also the costs of their implementation. It is in accordance with the state concept of the health care system of Georgia for 2014-2020, which includes the fundamental principles of the country's health sector development and is adjusted to the universal health care program, which is the pillar of the health care system development in the country and gives priority to the health of mothers and children.

The action plan is tailored to the country context and reflects the most urgent needs. It is harmonized with international strategic documents such as the UN Sustainable Development Goals and Targets (2015) and the World Health Organization (WHO) European Action Plan for Sexual and Reproductive Health Development (2017-2021).

While developing the action plan, other important international agreements and frameworks/strategies were taken into account: Program of Action of the International Conference on Population and Development (1994); Report of the Fourth World Conference on Women (1995); World Health Organization Global Strategy for Reproductive Health (2004); Report of the European Bureau of the World Health Organization - "Health 2019"; and finally, Global Strategy for Promoting the Health of Women, Children and Adolescents, 2017-2030 (2015).

The National Strategy and Action Plan is a guide for the government and all stakeholders in the development and implementation of policies and programs in the areas of maternal and newborn health, family planning and youth sexual and reproductive health.

The action plan details indicators for monitoring and evaluating progress achieved, and where possible and appropriate, provides specific milestones that are planned to be achieved for the completion of the action plan. The most important indicators are: maternal mortality rate, neonatal mortality rate, contraceptive prevalence rate, total abortion rate, unmet need for family planning and adolescent birth rate, etc.

8.2. guiding principles

The international agreements and strategic documents listed above emphasize the importance of principles related to the quality of interventions and human rights. These principles are:

- Human rights (including the right to health, non-discrimination, gender equality, solidarity, social justice, etc.).
- Evidence-based, results-oriented, affordable, high-quality services.
- Patient-centered, accessible, high-quality continuous service.
- Ensuring intersectoral cooperation and accountability to improve service quality and equity.
- More emphasis on prevention, promotion of healthy lifestyle, community involvement, expansion of their rights and opportunities.

8.3. Vision, purpose and strategic objectives

The vision of the current 2017-2019 action plan for the promotion of maternal and newborn health in Georgia (and the related long-term strategy) is to eliminate preventable cases of maternal and newborn deaths and stillbirths; All children will be wanted and all unwanted pregnancies will be prevented.

The strategy aims to maintain and expand evidence-based and effective interventions to protect maternal and newborn survival and health.

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In order to, As well as reproductive health directly related to them
Strengthening the scope and high quality services universal/universal
Ensuring accessibility.

8.4. strategic tasks

The action plan briefly outlines the directions, approaches and specifics
Priority interventions to be implemented by 2020 and which
It is necessary to achieve the set goal. The action plan includes three areas and the following
Tasks:

1. Promoting the health of mothers and newborns

task 1: By 2020, the availability of women will increase significantly
Evidence-based prenatal, antenatal, obstetric, neonatal and
on postnatal services and the use of these services will be expanded.

task 2: Quality of maternal and newborn healthcare services by 2020
Services will be improved, integrated and standardized.

Task 3 : By 2020, the awareness of the population will be significantly improved
Knowledge of healthy behavior, high quality medical service standards and patients
about rights.

2. Family Planning Youth and Sexual Reproductive Health

Task 4 : Target to significantly improve family planning services by 2020
for groups.

It should be especially noted that the combined interventions given in the action plan
The list will only partially contribute to the strategy envisaged
to fulfill long-term (2030) tasks. mentioned long-term tasks
Implementation requires additional interventions between 2020 and 2030. therefore
Some of the 2030 strategies for promoting maternal and newborn health
Intervention is not included in this action plan; Some interventions can be in this 3-year
within the framework of the action plan to start, but not finish; And some may start and
be finished.

Accomplishing the above-mentioned tasks requires measures and interventions
implementation at different system levels and using different approaches. basic
Strategic actions will be implemented in the following areas:

1. Leadership and governance
2. Service quality and availability
3. Human resources
4. Healthcare financing
5. Healthcare Management Information System
6. Community awareness.

8.5. Achievable targets for 2020 and 2030

Since this action plan covers only the first steps in the long term

To fulfill the tasks defined in the strategy, the main targets are given for the years 2020 and 2030.

- Reducing the maternal mortality rate from the current 32 per 100,000 live births to 25 in 2020, and to 12 in 2030.
- Reduce neonatal mortality from the current 6.1 per 1,000 live births to below 6 in 2020, and to 5 in 2030.
- Reducing the unmet need for family planning from the current 30.5% to 25% in 2020, and to 15% in 2030.
- Reducing the total abortion rate (the average number of abortions in a woman's lifetime) from the current 1.6 to 1.3 in 2020, and below 0.5 in 2030.
- Reducing the adolescent birth rate from the current 48.6 per 1,000 women aged 15-19 to below 40 in 2020, and below 20 in 2030.

8.6. Stakeholder Responsibilities and Functions

Implementation of this action plan requires joint efforts of various stakeholders. Below are some of them:

- Government of Georgia, Ministry of Labor, Health and Social Protection of Georgia, other ministries, Parliament of Georgia, local self-government bodies;
- LLP under the state control of the Ministry of Labor, Health and Social Protection of Georgia - L. Sakvarelidze Disease Control and Public Health Center;
- Social Services Agency under the state control of the Ministry of Labor, Health and Social Protection of Georgia;
- State regulatory agency for medical activities under the state control of the Ministry of Labor, Health and Social Protection of Georgia;
- non-governmental organizations;
- professional associations;
- Scientific-research and training institutions;
- media means;
- private sector;
- International donors.

8.7. Funding of measures to promote the health of mothers and newborns

The amount of money required for financing the measures of the 2017-2019 action plan for the implementation of the national strategy for the promotion of maternal and newborn health care of Georgia is 28,123,440 GEL (see detailed information in subsection 8.8.). Funds allocated from the state budget will finance 82% of the priority directions, which amounts to 23,158,225 GEL for three years.

Funds allocated by donor organizations cover only 8% of needs. The deficit needed to finance the measures envisaged by the action plan is 10% and represents a total of 2,676,000 GEL in 2017-2019.

8.8. Measures for the implementation of the National Strategy for the Promotion of Health Protection of Mothers and Newborns of Georgia for 2017-2019

Target 1: By 2020, significantly increase women's access to evidence-based and appropriate prenatal care, antenatal, obstetric, neonatal and postnatal services and will expand the use of these services.					
Outcome 1: Women and newborns use evidence-based prenatal, antenatal, obstetric, neonatal, and postpartum services					
Indicators:					
1: Percentage of women who received prenatal care services					
Baseline: N/A%; Target: 30%					
2: Percentage of women who attended at least one or more antenatal visits					
Basic: 92%; Target: y 95%					
3. Percentage of women who made a postpartum visit to a medical facility within 6 weeks after giving birth					
Basic: N/A ; Target: y 60%					
4. Percentage of newborns who received a pediatrician's consultation during the first week after discharge from the maternity hospital					
Baseline: N/A Target: y90%					
5. Percentage of out-of-pocket expenditures for reproductive/family planning, maternal, and newborn health (RMNCAH) services out of total out-of-pocket payments.					
Basic: 5.8%; Target: 4.8%					
intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	Budget, (GEL)
	1. 1.1.1 of Georgia is approved. 2017-2030 National Strategy for the Promotion of Maternal and Newborn Health 2017-2030 and the 2017-2019 Action Plan for its Implementation	Approval of the national strategy for the promotion of newborn health and the action plan for 2017-2019.			
	Basic: No	1.1.2. Develop and use evidence-based advocacy tools for members of parliament, central and local government, and other stakeholders to garner technical assistance, resource support, and social support for the maternal and newborn health care strategy and action plan.			1000
	Target: Yes	1.1.3. Review of the legal framework regulating the protection of mothers and children and their rights.			
		1.1.4. Strengthening collaborative partnerships and accountability to promote maternal and newborn health: 1.1.4.1. Involvement of relevant government agencies, local authorities and civil society in the implementation of the action plan; 1.1.4.2. Focusing all partners on the maternal and newborn health promotion strategy, raising awareness of appropriate actions with broad involvement of stakeholders in the accountability process; 1.1.4.2. Focusing all partners on the maternal and newborn health promotion strategy, raising awareness of appropriate actions with broad involvement of stakeholders in the accountability process; 1.1.4.3. Increasing the role and responsibility of the Maternal and Child Health Coordinating Council in improving inter-sectoral/agency coordination and cooperation; operational			13750

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1.1. Mechanisms supporting the environment necessary for the implementation of the national strategy for maternal and newborn health and family planning have been established.	2. Number of advocacy meetings held with the target audience								
	Basic: None								
	Target: 10								
</									

	3. The minimum package of primary reproductive health services is included in the emergency preparedness and response plans of the Ministry of Labor, Health and Social Protection of Georgia. Basic: No Target: Yes				
intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
studies have been developed and implemented to improve the quality of maternal and newborn health Mechanisms for reducing geographic, socio-cultural and financial barriers related to access to services	1. Conducted special 1.2. A number of Basic: No Target: At least one	1.2.1. Analysis of key challenges to implementing effective interventions to ensure high-quality maternal and newborn health care services (at least one special study).	sshjsds	2017-2019	7000
			LLC "L. Sakvarelidze National Center for Disease Control and Public Health"		
	2. The number of women and newborns (percentage) who received health care services (antenatal, postpartum, postnatal) at the apartment Basic: N/A	1.2.2. Development and implementation of residential maternal and newborn health care programs by strengthening the role of primary health workers in preventive, antenatal, antenatal, postnatal care and early childhood development, especially in rural/hard-to-reach areas:	UNICEF	2017-2019	1978750
		1.2.2.1. Develop and implement effective training programs and oversight mechanisms for primary health care providers to implement maternal and newborn medical services programs on the flat;	sshjsds DCSJ too		
		1.2.2.2. development and implementation of register of medical services programs for mothers and newborns in the apartment;			

1.2.2.3. To visit the apartment
Motivating primary care providers
with travel and transportation benefits.

intermediate result	intermediate result indicator	activity	Executor agency	deadlines	budget
1.3. Mother's sister of newborns healthcare services Availability in order to increase developed and A theme has been introduced level of mobilization and participation promotion Mechanisms	1. With the target audience Conducted benchmarking number of meetings Basic: None Target: 11	1.3.1. Their work with local authorities to ensure participation.	sshjsds DCSJ too	2017-2019	3125
		1.3.2. Public orientation and informing on maternal and newborn health needs, and linkages between families and the health system On strengthening: 1.3.2.1. Meetings aimed at raising awareness Conducting local self-government and with representatives of non-governmental organizations, with health workers and religious leaders to identify priority problems and means of solving them at the local level to find out.			15000

Task 2: By 2020, the quality of maternal and newborn health services will be improved and standardized for full integration of these services in parallel.

Final Outcome 2: The quality of maternal and newborn health services is improved and standardized according to the following indicators, which are included in the health management information system.

Indicators:

1. Percentage of pregnant women who had their first antenatal visit before 12 weeks of gestation

Basic: 92%; Target: ý95%

2. The percentage of pregnant women who Iron/folic acid supplements are taken at the recommended dose during pregnancy

Basic: 22% (2016); Target: ý95%

3. HIV-infection/AIDS prevalence among pregnant women (Classified by age groups)

basic:

age group	15-49	15-19	20-24	25-29	30-34	35-39	40-44	45-49
frequency (%)	0.1	0	0.1	0.1	0.1	0.1	0	0

Numerator: Number of pregnant women who were tested for HIV/AIDS and found to be HIV-positive (including women who already knew are HIV-positive) and who visited antenatal clinics.

Denominator: Number of women tested for HIV/AIDS at antenatal clinics (including women already known to be HIV-positive).

Target:

age group	15-49	15-19	20-24	25-29	30-34	35-39	40-44	45-49
frequency (%)	0.05	0	<0.05	<0.05	<0.05	<0.05	0	0

4. Prevalence of syphilis in pregnant women (Classified by age groups)

Basic:

frequency (%)	0.2
---------------	-----

There is no disaggregated data

Counter: Number of women who were tested for syphilis at antenatal clinics and received a positive result.

Denominator: Number of women tested for syphilis at antenatal clinics.

Target:

frequency (%)	0.1
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5. Percentage share of births in maternity hospitals with corresponding level of risk

Baseline: N/A Target: 90%

6. Maternal mortality due to specific causes (impact)

Basic: 57.9; Target: 90%

7. Neonatal mortality due to a specific cause (impact)

Basic: 98.1%; Target: 99%

Causes of neonatal mortality

8. Incidence of cases of congenital syphilis (in stillbirths, live births or loss of fetuses at the age of more than 20 weeks of gestation or less than 500g Weight of fetuses tested positive for syphilis of mothers who did not receive appropriate syphilis treatment) per 100,000 live births (impact)

Basic: 26.4

Numerator: The number of reported cases of congenital syphilis in a given year.

Denominator: number of live births

Target: 10 cases per 100,000 live births

9. New cases of pediatric HIV/AIDS mother-to-child transmission per 100,000 live births (impact).

Baseline: 1.7 (2015 1 case).

Target: 1 case per 100,000 live births.

10. Infant mortality rate in hospitals, classified by birth weight (influence

>4000 g, 2500–3999 g, 2000–2499 g, 1500–1999 g, 1000–1499 g, <1000 g

basic:

>4000 g	2500–3999 BC	2000-2499	1500-1999	1000–1499 AD	<1000 g
0.7	1.1	1.8	2.3	178.6	662.7

Target:

>4000 g	2500–3999 BC	1500-1999	1000–1499 AD	<1000 g
0.35	0.5	(20% off)	(15% reduction)	(5% reduction)

11. Stillbirth rate during childbirth (impact)

Basic: 14%; Target: 11%

12. Percentage share of cesarean births in the total number of births

Basic: 41.4%; Target: 31%

13. Frequency of caesarean section in low-risk parturients.

14. Prevalence of low birth weight among all live births.

Basic: 6.1%; Target: 5% reduction

15. The percentage of newborns on exclusive breastfeeding at the time of discharge from the maternity hospital.

Basic: 95%; Target: 98%

16. Percentage of newborns born in the maternity ward and transferred to the neonatal intensive care unit

Baseline: N/A Target: 18%

17. Percentage of pregnant women who are satisfied with the antenatal services received

Baseline: N/A Target: 80%

intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
		2.1.1. Integration of preconception services into the basic package of primary care: 2.1.1.1. Revision of financial mechanisms for integration of pre-conception services into the basic PJD package. 2.1.1.2. Strengthening the capacity of PJD providers to provide prenatal care (timely diagnosis and treatment of non-communicable and communicable diseases, provision of folic acid and iron supplements, counseling on healthy lifestyle, negative effects of tobacco, alcohol and drugs during pregnancy) through trainings conducted within the framework of the national strategy .	sshjsds LSI "Social Service Agency (SMS)" DCSJ too	2017-2019	217000
	1. Percentage of primary care facilities that provide preconception services as part of the basic care package. Basic: No Target: 15% antenatal services for childbirth according to the level assigned within regionalization.	2.1.2. Revising and updating the minimum package of antenatal care to ensure that women receive WHO-recommended visits and all evidence-based interventions, including detection and management of pre-existing conditions (non-communicable and communicable diseases), early detection and treatment of pregnancy complications, healthy lifestyles Health promotion and disease prevention (recognition of danger signs, proper nutrition with folic acid and iron supplements, 2. Percentage of institutions, change of risky behavior, etc.), preparedness that provide			
	Basic: None Target: y70%	2.1.3. Revision of financing mechanisms for the updated package of antenatal services.	sshjsds Professional associations		17302600
	3. Percentage of pregnant women who had preterm birth and received corticosteroids in the antenatal period. Basic: None Target: y90%	2.1.4. New guidelines for pregnancy and antenatal care prepared by the World Health Organization	sms	2017-2019	
	4. Percentage of pregnant women who were tested for HIV and received test results during pregnancy, during delivery and after delivery (within 72 hours of delivery), including pregnant women with previously detected HIV status. Baseline: 93.8% Target: y95%	Development of adapted learning internet modules, introduction of distance internet learning methods in the continuous medical education system.	State Regulatory Agency of Medical Activities (USSR) UNICEF UNFPA		37500
	5. The percentage of pregnant women	2.1.5. Development and implementation of an integrated plan for the elimination of congenital syphilis and mother-to-fetus HIV infection at the national level.			1309500

<http://www.matsne.gov.ge> 47000000010003020168

		2.1.9.1. Revision of postnatal care financing mechanisms; 2.1.9.2. Strengthening the connection to the early intervention program for the care of premature and premature newborns.	Professional associations		
		2.1.10. Promotion, support and protection of early and exclusive breastfeeding: 2.1.10.1. Assessment of the implementation of the Law of Georgia on the Promotion of Breastfeeding; 2.1.10.2. Development and implementation of law enforcement mechanisms.	sshjsds UNICEF DCSJ too	2017-2019	19600
		2.1.11. Forecasting the demand, procurement and supply of medicines for maternal and newborn care, especially folic acid, iron, micronutrient supplements and surfactants.	sshjsds service providers	2017-2019	1410100
intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
2.2. developed and is carried out by mothers and (percentage) life-and-death process improvement in both the public and private sectors.	Number of facilities and (percentage) that collect data. indicators. Baseline: 0 Target: 90% 2. Number of neonatal facilities that conduct a review/audit of maternal health death/ services. Quality improvement measures by providing advocacy, regulatory and reporting mechanisms Basic: 6 (5%) Target: 40% 3. Percentage of maternal and perinatal (institutional) deaths for which a death audit was conducted. base: 100% Target: 100%	2.2.1. Periodic review of guidelines and protocols for preconception, antenatal, obstetric, and postnatal/postnatal services in the 1. data. On selected perinatal service quality	Professional associations	2017-2019	
		2.2.2. Development and implementation of quality indicators of maternal and newborn health services: 2.2.2.1. selection of the basic package of indicators for maternal and newborn health services; 2.2.2.2. Development and implementation of standard operating procedures for collecting data on quality indicators of maternal and newborn health services; 2.2.2.3 Preparation of a report based on data analysis on the ranking of institutions, and notification of the results to all institutions, including through the website of the Ministry of Labor, Health and Social Protection of Georgia.	sshjsds sms DCSJ too	2017-2019	
		2.2.3. Standardization of Maternal and Neonatal Mortality/Life-Death Audit Practices: 2.2.3.1. Develop and implement standard operating procedures for auditing medical records in maternal and newborn health care providers; 2.2.3.2. Communication of audit results and recommendations to perinatal facilities, and dissemination of good practices among them.	sshjsds UNFPA Professional associations	2017-2019	50000
intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
2.3. Policies have been developed to help Target: Yes maintain qualified health personnel as needed.	1. A policy has been developed on the introduction of a mandatory continuous medical education and a system of re-certification of doctors. Basic: No generate, retain, and	2.3.1. Development of a policy in connection with the introduction of a mandatory continuous medical education and certification system for doctors in the future 2.3.1.1: Review/draft and approve relevant regulations, including: • updating of basic competencies; • Development/revision of accreditation criteria and Continuing Medical Education (CME) policy;	sshjsds	2017-2019	

Distribution and motivation.	Continuing medical education mandatory course.	• Development of the credit-score system; • development/revision of commercial support standards for compulsory USG; • Development/revision of requirements and institutional mechanisms for the administration of the mandatory USG program. 2.3.1.2. Facilitation of the development and implementation of mandatory USG courses by professional associations.	USSR Professional associations		
Intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
2.4. Innovative mechanisms for financing maternal and newborn services have been introduced.	(percentage) of antenatal facility-based and implementation of antenatal payment systems (P4P) that are involved in the development of antenatal services in selected regions to improve outcomes/indicators. Based on the payment (P4P: pay for performance) system	2.4.1. Outputs/indicators 1. Number of antenatal services in selected regions to improve outcomes/indicators. Based on the payment (P4P: pay for performance) system	sshjsds sms	2017-2019	22500
	Baseline: 0 Target: TBD	2.4.2. Development and implementation of a diagnosis-related group (DRG) system in perinatal and neonatal care services.			147500
intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
2.5. To improve the quality of data for making evidence-based decisions about maternal and newborn health	1. The number (percentage) of institutions that have implemented a data quality audit system. Baseline: 0 Target: TBD	2.5.1. Development and implementation of a data quality audit system at the national and institutional levels based on the assessment of various maternal and newborn health data registries: 2.5.1.1. development/adaptation of a routine data quality assessment tool; 2.5.1.2. Monitoring and verification (recalculation) of the results of the selected indicators based on the routine data quality assessment protocol; 2.5.1.3. Sharing audit results and recommendations with relevant institutions.	DCSJ too UNICEF DCSJ too	2017-2019	76500
	2. Number of conducted special studies. Baseline: 0 Target: 1	2.5.2. Facilitate data collection through targeted research, including household surveys, facility evaluations, operational studies, and other means of examining processes and outcomes related to maternal and newborn health.			1500000
Mechanisms for strengthening the health care management information system have been developed and implemented.	3. Number (percentage) of staff trained in use of death certification and ICD-10 coding. Basic: 10% Target: 25%	2.5.3. Improving the completeness and quality of registration of deaths in the civil registry and other information sources, including: 2.5.3.1. Develop on-site training programs for cause of death certification and ICD-10 coding for civil registry and and implement a code demographic statistics system for assigners, data providers, statisticians, etc. to standardize and improve the quality of cause of death determination and coding practices.	DCSJ too	2017-2019	105000
		2.5.4. Strengthening surveillance and response to maternal and perinatal deaths, including timely reporting of all maternal and perinatal deaths. 24 during the hour)			

Task 3: By 2020, the awareness and knowledge of the population about healthy behavior, high-quality medical service standards and patients' rights will be significantly improved.

Final result 3: The population's knowledge about maternal and newborn health care services, attitude towards this service and its practical use are improved.

Indicators:

1. Percentage of women of reproductive age who can recognize maternal danger signs.

Baseline: N/A Target: 40%

2. Percentage of women of reproductive age who can recognize danger signs of a newborn.

Baseline: N/A Target: 40%

intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
3.1. Developed and viewed program implemented health. Communication promotion mechanisms related to communication and behavior change among mothers and fathers To raise awareness of the population about the health of newborns.	1. Percentage of target audience that has heard/ messages to inform, educate, and promote maternal and newborn communication promotion mechanisms related to communication and behavior change among mothers and fathers Basic: None Target: 90%	3.1.1 Prepare and conduct a communication campaign related to information, education, communication and behavior change in order to improve the healthy behavior of mothers and families, including seeking services during uncomplicated pregnancy and childbirth, and seeking emergency treatment in case of complications: 3.1.1.1. Preparation and refinement of the most important messages and communication materials related to information, education, communication and behavior change on specific issues of maternal and newborn health for the population (women, men and adolescents), with an emphasis on the following issues: · Advantages of applying to medical institutions at an early stage; Preparation for childbirth; · Nutritional practices important for the health of mothers and newborns;; postpartum/postnatal care and newborn care; · causes of death of mothers and newborns and recognition of danger signs; · The role of men in protecting the health of mothers and newborns. 3.1.1.2. Disseminating key messages about maternal and newborn health and promoting healthy behaviors using multifaceted approaches (Television, radio, social media, mobile phones, billboards, print media, information, education and communication materials, etc. and)	DCSJ too	2017-2019	151000
		3.1.2 Conducting an impact assessment of the communication campaign produced by various media means.			
intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
		3.2.1. Engaging the private sector in promoting zero tolerance for preventable deaths and advocating optimal behavior: 3.2.1.1. Provision of private sector providers for healthy lifestyle promotion and behavior change activities	sshjsds DCSJ too	2017-2019	

3.2. Mechanisms have been established to strengthen advocacy and social mobilization activities to support healthy behaviors of mothers and their family members and maternal and newborn programs.	1. 3.2.2 of measures dedicated to maternal and newborn health issues. Develop and use advocacy tools for dialogue with stakeholders, professional associations, as well as state and municipal level policy makers and decision makers. Baseline: 0 Target: 1	with messages and materials; 3.2.1.2. Orientation of healthcare workers and managers on the use of informational, educational and communication materials.	Private sector service providers		
		tools for dialogue with stakeholders, professional associations, as well as state and municipal level policy makers and decision makers, including religious leaders, to promote maternal and newborn health communication activities, including resource allocation.	sshjsds	2017-2019	3000
		3.2.3. Developing a press kit and conducting awareness raising seminars for representatives of central and local media (radio, television, newspapers) in order to increase the coverage of maternal and newborn health programs and related issues in relevant media and to disseminate successful examples.	DCSJ too	2017-2019	25000
		3.2.4. Organization of social mobilization events, including Mothers' and Newborns' Week and celebration of International Women's and Children's Days.	sshjsds DCSJ too local government non-governmental organizations Donors	2017-2019	137500

Task 4: By 2020, family planning services for target groups will be significantly improved.

Final Outcome 1: By 2020, access to family planning services for target groups will be significantly increased

These ultimate outcomes will have a positive impact on reducing maternal and neonatal morbidity and mortality rates

Indicators:

1: Percentage of women of reproductive age (15-49 years) using a modern method of contraception:

Baseline: 35% (Reproductive Health Survey-2010); Target: 45% (2018, multi-indicator cluster survey).

2: Total rate of induced abortions

Basic: 1.6 (Reproductive Health Research -2010); Target: 1.4 (A multi-indicator cluster study -2018).

intermediate result	Intermediate result indicator	activity	implementing agency	deadlines	budget
4.1. The leadership role of the state in the effective leadership, management and coordination of the family planning (FP) program is strengthened.	1. Family planning counseling and provision of contraceptives are included in the basic package of universal health care for selected target groups. Basic: No (2016) Target: Yes (2020)	4.1.1. Development of appropriate methodology to identify the target group of beneficiaries, and budget analysis to decide on the provision of free contraceptives to youth and beneficiaries of the target program of social assistance.	sshjsds sms	2017-2020	
		4.1.2. Integrating OD services and modern methods of contraception into universal health care in the Basic Program Package (BBP) for the target group of beneficiaries.			2875000
		4.1.3. Revision of current legislation in order to create a favorable legal framework for the inclusion of OD services at the primary health care level.			
	2. Basic The medication list includes contraceptives pre-classified by WHO.				

	Basic: No (2016) Target: Yes (2020)	4.1.4. Preparing a list of modern methods of contraception			
4.2. The role of the state in raising awareness and improving knowledge of sexual and reproductive health among young people is strengthened.	3. Number of school doctors who were trained in Kakheti and Samegrelo regions. Baseline: 0 (2016) Target: 60 (2018)	4.2.1. Training of school doctors on sexual and reproductive health of young people according to accredited training programs in this field. 4.2.2. Development of training materials on healthy lifestyle and donor organizations further inclusion in the formal education system.	DCSJ too UNFPA Ministry of Education and Science	2017-18	40 000 2500
family planning data collection services have been developed for delivery, regulation and governance mechanisms at each level.	1. Family planning protocols and standard operating procedures for primary care have been developed and adopted. Basic: No (2016) Target: Yes (2020) 2. Developed with the use of family planning services 4.3. Linked high-quality collection services tools have been developed for delivery, regulation and governance mechanisms. Basic: No (2016) Target: Yes (2020) 3. A multi-indicator cluster study and reproductive health study of mothers and fathers have been conducted To collect data on newborn health and reproductive health promotion strategies. Basic: No (2016) Target: Yes (2018)	4.3.1. Development and implementation of OD protocol and standard operating procedures within the PJD based on the recommendations of the World Health Organization. 4.3.2. Develop a minimum data collection tool for each level of service to routinely collect and analyze data on service quality and utilization. 4.3.3. Strengthening of regulatory mechanisms to ensure the quality of OD.	sshjsds DCSJ too UNICEF UNFPA	2017-2020	7500
4.4. Increased skills and knowledge of health workers in family planning services	1. Revised 4.4.1 of training of family doctors. Renewal of undergraduate and postgraduate education and standardized continuing medical education curricula, including curricula for nurses. in order to Ensure proper inclusion of the OD component and the full range of contraceptive methods.	4.4.2. Revision of existing training curriculum and modules for reproductive health services	sshjsds Tbilisi State Medical University Professional associations	2017-2018	

	Basic: No (2016)	for providers and family physicians to ensure that these programs include a comprehensive family planning component.	UNFPA		
	Target: Yes (2018)				
evidence-based strategies for family planning young people. services have increased, including among people	Basic: No (2016) Target: Yes (2018)	4.5.1. Facilitating the development of awareness-raising strategies and messages based on the principle of engagement;			
		4.5.2. Strengthening dialogue with media representatives and journalists about the benefits of family planning and ways of media coverage.	sshjsds		
		4.5.3. Initiate multimedia campaigns through television, radio, newspapers, magazines and specially prepared information booklets.	DCSJ too	2017-2020	
		4.5.4. Preparation of booklets on educational tools, visual aids and methods for beginners to use these methods in order to distribute them in medical institutions.	Professional associations UNFPA		5400
		4.5.5. Build partnerships with stakeholders and civil society organizations to promote OD and increase access to youth-friendly reproductive health/family planning services and information.			
Total					28123440

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